CONSOLIDATED PLAN DOCUMENT AND SUMMARY PLAN DESCRIPTION
Good Health Distribution Partners, LLC

SEC 1: INTRODUCTION

1.1 Plan creation and adoption of documents: This document constitutes a combined Plan Document and Summary Plan Description as required by Section 102 of the Employee Retirement income Security Act of 1974, as amended, ("ERISA") and US Department of Labor Regulation Sections 2520.102.2 and 2520 102-3 for the Plan. This plan is effective the 1st of July, 2024. The purpose of this plan is to provide certain medical benefits to you and your dependents as defined below.

1.2 Access To Plan Documents: You can obtain copies of all Plan documents and other Plan information including provider contracts and collective bargaining agreements, and copies of the latest annual report (Form 5500 Series) and updated summary plan description upon written request to the Plan Administrator. The Plan Administrator may make a reasonable charge for the copies. You may also examine these documents, without charge, at the Plan Administrator's office and at other specified locations.

1.3 Choice of Language: if you require this notice in another language, please forward a written request to: accountservices@mbaadmin.com.

1.4 Basic Plan Info:

- 1.4.1 Plan Name: Good Health Distribution Partners LLC
- 1.4.2 Employer EIN: 84-3051226
- 1.4.3 Plan Number: 501843051226
- 1.4.4 Type of Plan: Employee Welfare Benefit Plan

1.4.5 Plan Administrator (Third Party) Merchants Benefit Administration "MBA"

1.4.6 Agent for Service: MBA 18700 N Hayden Rd Suite 390, Scottsdale, AZ 85255

1.4.7 Source of Funding: Self-Funded, PROVIDED HOWEVER nothing herein will be construed to require the Company, Sponsor or Plan Administrator to maintain any fund or segregate any amount for the benefit of any Participant in this Plan, and no Participant or other person shall have any claim against, right to, or security or other interest in, any fund, account or asset of the company from which any self-funded benefit payment under the Plan may be made.

1.4.8 Plan Year: July 1, to June 30

SEC 2: DEFINITIONS

2.1. ACA: Collectively, The Patient Protection and Affordable Care Act of 2010 and the Health Care and Education Reconciliation Act of 2010 as amended, including and regulations or guidance thereunder.

2.2 Actively at work or active employment: An Employee is "Actively at Work" or in "Active Employment" on any day the Employee performs in the customary manner all of the regular duties of employment. An Employee will be deemed Actively at Work on each day of a regular paid vacation or on a regular non-working day on which the covered Employee is not totally disabled, provided the covered Employee was Actively at Work on the last preceding regular work day. An Employee shall be deemed Actively at Work if the Employee is absent from work due to a health factor, as defined by HIPAA, subject to the Plan's Leave of Absence provisions. An Employee will not be considered under any circumstances Actively at Work if he or she has effectively terminated employment.

2.3 Adverse Benefit Determination: means any of the following:

2.3.1 A denial in benefits.

2.3.2 A reduction in benefits.

2.3.3 A rescission of coverage, even if the rescission does not impact a current claim for benefits.

2.3.4 A termination of benefits.

2.3.5 A failure to provide or make payment (in whole or in part) for a benefit, including any such denial, reduction, termination, or failure to provide or make payment that is based on a determination of a Claimant's eligibility to participate in the Plan.

2.3.6 A denial, reduction, or termination of, or a failure to provide or make payment (in whole or in part) for a benefit resulting from the application of any utilization review.

2.3.7 A failure to cover an item or service for which benefits are otherwise provided because it is determined to be Experimental or Investigational or not Medically Necessary or appropriate.

2.4 Allowable Expenses: Allowable Expense(s) shall mean the Maximum Allowable Charge for any eligible item of expense, at least portion of which is covered by the plan.

2.5 Alternate Recipient: means any Child of a Participant who is recognized under a Medical Child Support Order as having a right to enrollment under this Plan as the Participant's eligible Dependent. For purposes of the benefits provided under this Plan, an Alternate Recipient shall be treated as an eligible Dependent, but for purposes of the reporting and disclosure requirements under ERISA, an Alternate Recipient shall have the same status as a Participant.

2.6 Annual Enrollment Period: the period of time set by the Plan to accept routine enrollments.

2.7 Assignment of Benefits: means an arrangement whereby the Participant, at the discretion of the Plan Administrator, assigns the right to seek and receive payment of eligible Plan benefits, less Deductibles, Copayments and the Coinsurance percentage that is not paid by the Plan, in strict accordance with the terms of this Plan Document, to a Provider. If a Provider accepts said arrangement, Providers' rights to receive Plan benefits are equal to those of a Participant and are limited by the terms of this Plan Document. A Provider that accepts this arrangement indicates

acceptance of an "Assignment of Benefits" and Deductibles, Copayments and the Coinsurance percentage that is the responsibility of the Participant, as consideration in full for services, supplies, and/or treatment rendered. The Plan Administrator may revoke or disregard an Assignment of Benefits previously issued to a Provider at its discretion and continue to treat the Participant as the sole beneficiary.

2.8 Calendar Year: the 12-month period from January 1st through December 31 of each year.

2.9 CDC: means the Centers for Disease Control and Prevention.

2.10 Child/Children: means the Employee's natural Child, any stepchild, legally adopted Child, or any other Child for whom the Employee has been named legal guardian, or an "eligible foster child," which is defined as an individual placed with the Employee by an authorized placement agency or by judgment, decree or other order of a court of competent jurisdiction. For purposes of this definition, a legally adopted Child shall include a Child placed in an Employee's physical custody in anticipation of adoption. "Child" shall also mean a covered Employee's Child who is an Alternate Recipient under a Qualified Medical Child Support Order, as required by the Federal Omnibus Budget Reconciliation Act of 1993.

2.11 CHIP: means the Children's Health Insurance Program or any provision or section thereof, which is herein specifically referred to, as such act, provision or section may be amended from time to time.

2.12 Claimant: means Participant of the Plan, or entity acting on his or her behalf, authorized to submit claims to the Plan for processing, and/or to appeal an Adverse Benefit Determination.

2.13 Clean Claim: means one that can be processed in accordance with the terms of this document without obtaining additional information from the service Provider or a third party. It is a claim which has no defect or impropriety. A defect or impropriety shall include a lack of required sustaining documentation as set forth and in accordance with this document, or a particular circumstance requiring special treatment which prevents timely payment as set forth in this document, and only as permitted by this document, from being made. A Clean Claim does not include claims under

investigation for fraud and abuse or claims under review for Medical Necessity or other coverage criteria, or fees under review for application of the Maximum Allowable Charge, or any other matter that may prevent the charge(s) from being Covered Expenses in accordance with the terms of this document.

2.14 CMS: means the Centers of Medicare and Medicaid Services

2.15 COBRA: means the Consolidated Omnibus Budget Reconciliation Act of 1985, as amended

2.16 COBRA Administrator: means the party who administers COBRA continuation as identified in Section 9.

2.17 Coinsurance: means a cost sharing feature of many plans. It requires a Participant to pay out-of-pocket a prescribed portion of the cost of Covered Expenses. The defined Coinsurance that a Participant must pay out-of-pocket is based upon his or her health plan design. Coinsurance is established as a predetermined percentage of the Maximum Allowable Charge for covered services and usually applies after a Deductible is met in a Deductible plan.

2.18 The Company: means (company name) and any successor to all or a major portion of its assets or business that assumes the obligations of (company name) under the plan.

2.19 Copayment or Copay: means the dollar amount a Participant pays for healthcare services at the time of delivery.

2.20 Covered Expenses: means a service or supply provided in accordance with the terms of this document, whose applicable charge amount does not exceed the Maximum Allowable Charge for an eligible service, treatment or supply, meant to improve a condition or Participant's health, which is eligible for coverage in accordance with this Plan. When more than one treatment option is available, and one option is no more effective than another, the Covered Expense is the least costly option that is no less effective than any other option.

2.21 Covered Person: means an enrolled Eligible Employee (Participant) as well as any Eligible Dependent or beneficiary who is or becomes covered under the

Plan. Your "Eligible Dependents (must be US citizens or legal residents and] and generally are:

2.21.1 Your spouse (defined as an individual lawfully married to you in any state, whether the spouse is of the opposite or the same sex as you);

2.21.2 Your eligible domestic partner; as determined pursuant to the Company's domestic partnership policies;

2.21.3 Your eligible domestic partner's eligible dependents; as determined pursuant to the Company's domestic partnership policies;

2.21.4 Any child of a covered Eligible Employee who has not attained age 26. A "child" of an Eligible Employee includes a biological child, stepchild, legally adopted child, or foster child placed with the eligible employee by judgment, decree, or other order of any court of competent jurisdiction. A "child" does not include the spouse or children of an Eligible Employee's child.

2.21.5 You are an " Eligible Employee" if: You work in the United States for the Company, and You meet any other eligibility requirements for the Plan, as set forth in Appendix A: Summary of Benefits, and you are legally authorized to work in the United States for the Company

2.21.6 You are not an " Eligible Employee" if you are: Engaged under an agreement that states you are not eligible to participate in the Plan; or a nonresident alien performing services outside the United States; or classified by the Company as a consultant; or employed by a staffing firm.

2.22 Dependent: means one or more of the following person(s):

2.22.1 An Employee's present spouse, thereby possessing a valid marriage license, not annulled, or voided in anyway.

2.22.2 An Employee's opposite-sex domestic partner who has the same principal place of abode for more than one half of the Calendar Year, and who relies on the Employee for more than one half of his or her support for the Calendar Year in which the domestic partner is enrolled for coverage under the Plan.

2.22.3 An Employee's Child who is less than 26 years of age. NOTE: Coverage for a Dependent Child will continue until the end of the calendar month he or she turns 26 years of age.

2.22.4 An Employee's Child, regardless of age, who is mentally or physically incapable of sustaining his or her own living. Written proof of such incapacity and dependency satisfactory to the Plan must be furnished and approved by the Plan within 31 days after the date the Child attains the limiting age as stated in the numbers above. The Plan may require, at reasonable intervals, subsequent proof satisfactory to the Plan during the next two years after such date. After such two-year period, the Plan may require such proof, but not more often than once each year.

2.22.5 Active-duty members of any armed force shall not be deemed to be "Dependents."

2.22.6 Residents of a country other than the United States shall not be deemed to be "Dependents."

2.22.7 To establish a Dependent relationship, the Plan reserves the right to require documentation satisfactory to the Plan Administrator.

2.23 Drug: means mean insulin and prescription legend drugs. A prescription legend drug is a Federal legend drug (any medicinal substance which bears the legend: "Caution: Federal law prohibits dispensing without a prescription") or a State restricted drug (any medicinal substance which may be dispensed only by prescription, according to State law) and which, in either case, is legally obtained from a licensed drug dispenser only upon a prescription of a currently licensed Physician.

2.24 Employee: any W2 employee, 1099 independent affiliate or GHD Limited Partner that promotes the business purposes of the Partnership. Who either: works on average at least 20 hours per week or at least 80 hours per month providing personal services to the working owner's trade or business, or has wages or self-employment income from such trade or business that at least equals the working owner's cost of coverage for participation by the working owner and any covered beneficiaries in the group health plan sponsored by the group or association in which the individual is participating, or any other indica of active engagement in furtherance of the Company's interests that the Sponsor deems

appropriate.

2.25 Employer: Good Health Distribution Partners LLC

2.26 ERISA: means the Employee Retirement Income Security Act of 1974 as amended.

2.27 Exclusion: means conditions or services the Plan does not cover.

2.28 Explanation of Benefits (EOB): means a statement a health plan sends to a Participant which shows charges, payments and any balances owed. It may be sent by mail or e-mail. An Explanation of Benefits may serve as an Adverse Benefit Determination.

2.29 External Review: means an independent review, by contracted third-party, of an Adverse Benefit Determination.

2.30 Family Unit: means the Employee and his or her dependents.

2.31 FDA: means the Food and Drug Administration.

2.32 Final Internal Adverse Benefit Determination: means an Adverse Benefit Determination that has been upheld by the Plan at the conclusion of the internal claims and appeals process, or an Adverse Benefit Determination with respect to which the internal claims and appeals process has been deemed exhausted.

2.33 FMLA: means the Family and Medical Leave Act of 1993, as amended.

2.34 FMLA Leave: means an unpaid, job protected Leave of Absence for certain specified family and medical reasons which the Company is required to extend to an Eligible Employee under the provisions of the FMLA.

2.35 GINA: means the Genetic Information Nondiscrimination Act of 2008, as amended

2.36 HIPAA: means the Health Insurance Portability and Accountability Act of 1996, as amended from time to time.

2.37 HRSA: means the Health Resources and Services Administration.

2.38 Increased Cost Exemption: means an exemption from the Mental Health Parity Act as described in Section 10 due to increased costs of providing such coverage in accordance with Section 712 c 2 of ERISA.

2.39 Incurred: means the date on which the service is rendered, or the supply is obtained.

2.40 Leave of Absence: means a period of time during which the Employee must be away from his or her primary job with the Employer, while maintaining the status of Employee during said time away from work, generally requested by an Employee and having been approved by his or her

Participating Employer, and as provided for in the Participating Employer's rules, policies, procedures and practices where applicable.

2.41 Legal Separation: means an arrangement to remain married but live apart, following a court order.

2.42 Maximum Allowable Charge: means the benefit payable for a specific coverage item or benefit under the Plan. The Maximum Allowable Charge will always be a negotiated rate if one exists; if no negotiated rate exists, the Maximum Allowable Charge will be determined and established by the Plan, at the Plan Administrator's discretion, using normative data and submitted information such as, but not limited to, any one or more of the following, in the Plan Administrator's discretion: Medicare reimbursement rates (presently utilized by the Centers for Medicare and Medicaid Services ["CMS"]), or Prices established by CMS utilizing standard Medicare Payment methods and/or based upon supplemental Medicare pricing data for items Medicare doesn't cover based on data from CMS), or Prices established by CMS utilizing standard Medicare payment methods and/or based upon prevailing Medicare rates in the community for non-Medicare facilities for similar services and/or supplies provided by similarly skilled and trained providers of care) or, Prices established by CMS utilizing standard Medicare payment methods for items in alternate settings based on Medicare rates provided for similar services and/or supplies paid to similarly skilled and trained providers of care in traditional settings) or Medicare cost data as reflected in the applicable individual provider's cost report(s) or The fee(s) which the Provider most frequently charges the majority of patients for the service or supply, or amounts the Provider specifically agrees to accept as payment in full either through direct negotiation or through a preferred provider organization (PPO) network or, Average wholesale price (AWP) and/or manufacturer's retail pricing (MRP) or, Medicare cost-to-charge ratios or other information regarding the actual cost to provide the service or supply or, the allowable charge otherwise specified within the terms of this Plan or, the prevailing range of fees charged in the same "area" (defined as a metropolitan area, county, or such greater area as is necessary to obtain a

representative cross-section of Providers, persons or organizations rendering such treatment, services, or supplies for which a specific charge is made) by Providers of similar training and experience for the service or supply. The Plan Administrator may in its discretion, taking into consideration specific circumstances, deem a greater amount to payable than the lesser of the aforementioned amounts. The Plan Administrator may take any or all of such factors into account but has no obligation to consider any particular factor. The Plan Administrator may also account for unusual circumstances or complications requiring additional or a lesser amount of time, skill and experience in connection with a particular service or supply, industry standards and practices as they relate to similar scenarios, and the cause of Injury or Illness necessitating the service(s) and/or charge(s). In all instances, the Maximum Allowable Charge will be limited to an amount which, in the Plan Administrator's discretion, is charged for services or supplies that are not unreasonably caused by the treating Provider, including errors in medical care that are clearly identifiable, preventable, and serious in their consequence for patients. A finding of Provider negligence and/or malpractice is not required for services or fees to be considered ineligible pursuant to this provision. The determination that fees for services are includable in the Maximum Allowable Charge will be made by the Plan Administrator, taking into consideration, but not limited to, the findings and assessments of the following entities: (a) The national medical associations, societies, and organizations; and (b) The Food and Drug Administration (FDA). To be includable in the Maximum Allowable Charge, services and fees must be in compliance with generally accepted billing practices for unbundling or multiple procedures. The Plan Administrator has the discretionary authority to decide if a charge is covered under this Plan. The Maximum Allowable Charge will not include any identifiable billing mistakes including, but not limited to, up-coding, duplicate charges, and charges for services not performed.

2.43 Medical Child Support Order: means mean any judgment, decree or order (including approval of a domestic relations settlement agreement) issued by

a court of competent jurisdiction that meets one of the following requirements:

Provides for child support with respect to a Participant's Child or directs the Participant to provide coverage under a health benefits plan pursuant to a State domestic relations law (including a community property law), or Is made pursuant to a law relating to medical child support described in §1908 of the Social Security Act (as added by Omnibus Budget Reconciliation Act of 1993 §13822) with respect to a group health plan.

2.44 Medicare: means the Federal program by which health care is provided to individuals who are 65 or older, certain younger individuals with disabilities, and individuals with End-Stage Renal Disease, administered in accordance with parameters set forth by the Centers for Medicare and Medicaid Services (CMS) and Title XVIII of the Social Security Act of 1965, as amended, by whose terms it was established.

2.45 MHPA: means the Mental Health Parity Act of 1996, as amended from time to time.

2.46 National Medical Support Notice or NMSN: means a notice that contains all of the following information: The name of an issuing State child support enforcement agency and, The name and mailing address (if any) of the Employee who is a Participant under the Plan or eligible for enrollment and, the name and mailing address of each of the Alternate Recipients (i.e., the Child or Children of the Participant) or the name and address of a State or local official may be substituted for the mailing address of the Alternate Recipients(s), the identity of an underlying child support order.

2.47 Non-Network or Out-of-Network: means the facilities, Providers and suppliers that do not have an agreement with a designated Network to provide care to Participants.

2.48 Open Enrollment Period: means the time frame specified by the Plan Administrator.

2.49 Participant: means any W2 Employee, 1099 Independent affiliate, Limited Partner or Dependent who is eligible for benefits (and enrolled) under the Plan.

2.50 Patient Protection and Affordable Care Act (PPACA): means the health care reform law enacted in March 2010, Public Law 111-148; PPACA, together with the Health Care and Education Reconciliation Act, is commonly referred to as Affordable Care Act (ACA). (See "Affordable Care Act").

2.51 PHI: means protected health information as defined in 45 CFR Sec. 165.501

2.52 Physician: means a Doctor of Medicine (M.D.), Doctor of Osteopathy (D.O.), Doctor of Dental Surgery (D.D.S.), Doctor of Podiatry (D.P.M.), Doctor of Chiropractic (D.C.), Psychologist (Ph.D.), Audiologist, Certified Nurse Anesthetist, Licensed Professional Counselor, Licensed Professional Physical Therapist, Master of Social Work (M.S.W.), Occupational Therapist, Physiotherapist, Speech Language Pathologist, psychiatrist, midwife, and any other practitioner of the healing arts who is licensed and regulated by a State or Federal agency, acting within the scope of that license.

2.53 Plan: means the (name of Plan) as set forth in this plan document and summary plan description, (including any and all amendments and supplements hereto.)

2.54 Plan Administrator: means the third-party administrator, or such other person or entity, as may be appointed from time to time by the Company to supervise the administration of the Plan.

2.55 Plan Year: means a period commencing on the Effective Date or any anniversary of the adoption of this Plan and continuing until the next succeeding anniversary.

2.56 Pre-Existing Condition: a Bodily Injury or Sickness, whether a physical or mental condition regardless of the cause for the condition, for which medical advice, diagnosis, care or treatment was recommended or received by an Insured Person within the Twelve [12] months immediately before that person's Effective Date, or for which symptoms of a condition have occurred that would have led a prudent person to seek medical advice, diagnosis, care, or treatment during the Twelve [12] immediately before that person's Effective Date. Medical advice, diagnosis, care or treatment will be taken into account only if it is recommended by, or received from an individual licensed or similarly authorized to provide such services under state law and

operating within the scope of practice authorized by state law. Medical treatment includes prescription medications.

2.57 Preventive Care: means certain Preventive Care services. To comply with the ACA, and in accordance with the recommendations and guidelines, plans shall provide In-Network coverage for all of the following: Evidence-based items or services rated A or B in the United States Preventive Services Task Force recommendations or, Recommendations of the Advisory Committee on Immunization Practices adopted by the Director of the Centers for Disease Control and Prevention, Comprehensive guidelines for infants, children, and adolescents supported by the Health Resources and Services Administration (HRSA). If a recommendation or guideline does not specify the frequency, method, treatment, or setting for the provision of that service, the Plan can use reasonable medical management techniques to determine any coverage limitations. Copies of the recommendations and guidelines may be found here: http://www.uspreventiveservicestaskforce.org or at https://www.healthcare.gov/coverage/preventive-care-benefits/. For more information, Participants may contact the Plan Administrator/ Employer.

2.58 Prior Plan: means the coverage provided on a group or group type basis by the group insurance policy, benefit plan or service plan that was terminated on the day before the Effective Date of the Plan and replaced by the Plan.

2.59 Prior to Effective Date or After Termination Date: means are dates occurring before a Participant gains eligibility from the Plan, or dates occurring after a Participant loses eligibility from the Plan, as well as charges Incurred prior to the effective date of coverage under the Plan or after coverage is terminated, unless continuation of benefits applies.

2.60 Privacy Standards: means the standards of the privacy of individually identifiable health information, as pursuant to HIPAA.

2.61 Provider: means an entity whose primary responsibility is related to the supply of medical care. Each Provider must be licensed, registered, or certified by the appropriate State agency where the medical care is performed, as required by that State's law where applicable. Where there is no applicable State agency, licensure, or regulation, the Provider must be

registered or certified by the appropriate professional body. The Plan Administrator may determine that an entity is not a "Provider" as defined herein if that entity is not deemed to be a "Provider" by the Centers for Medicare and Medicaid Services (CMS) for purposes arising from payment and/or enrollment with Medicare; however, the Plan Administrator is not so bound by CMS' determination of an entity's status as a Provider.

2.62 Qualified Medical Child Support Order or QMCSO: means a Medical Child Support Order, in accordance with applicable law, and which creates or recognizes the existence of an Alternate Recipient's right to, or assigns to an Alternate Recipient the right to, receive benefits for which a Participant or eligible Dependent is entitled under this Plan.

2.63 Security Standards: means the final rule implementing HIPAA's Security Standards for the Protection of Electronic Protected Health Information (PHI), as amended.

2.64 Service Waiting Period: means an interval of time that must pass before an Employee or Dependent is eligible to enroll under the terms of the Plan. The Employee must be a continuously Active Employee of the Employer during this interval of time.

2.65 SSA: means the United States Social Security Administration

2.66 Third Party Administrator: means the person or entity to which the Company has delegated the day-to-day functions of the Plan to. See also: "Plan Administrator".

2.67 Uniformed Services: means the Armed Forces, the Army National Guard and the Air National Guard, when engaged in active duty for training, inactive duty training, or full time National Guard duty, the commissioned corps of the Public Health Service, and any other category of persons designated by the President of the United States in time of war or emergency.

2.68 USERRA: means the Uniformed Services Employment and Reemployment Rights Act of 1994 ("USERRA").

SEC 3: ADMINISTRATION

3.1 Administration: The plan is sponsored by and administered by the Company. The Company may delegate all or any portion of its authority to any person or entity.

3.2 Plan Administrator: The Company has contracted with a Third-Party Administrator ("Plan Administrator") and has delegated the performance of all the day-to-day operations of the Plan to it. The functions and authority of the Plan Administrator include, but are not limited to:

3.2.1 Interpretation of the Terms of the Plan

3.2.2 Determination of factual questions that arise in the course of administering the plan

3.2.3 To adopt rules and regulations regarding the administration of the Plan, including enrollment procedures

3.2.4 The determine the conditions under which benefits become payable under the Plan

3.2.5 To make determinations of eligibility under the Plan

3.2.6 To verify the initial and continuing eligibility for participation and benefits under the Plan of any person, including any child, spouse, domestic partner, or dependent of an employee, by requesting proof of such eligibility including, as applicable and with limitation, tax returns, business records, cell phone and email records, marriage certificates, birth certificates, proof of residence, proof of domestic partnership, or other documentation deemed appropriate by the Third Party Administrator, to make any other determinations that the Third Party Administrator believes are necessary and advisable for the administration of the Plan.

3.2.7 Claims Processing

3.2.8 Claims Adjudication i.e. approval/denial

3.2.9 Appeal process

3.2.10 Government reporting related to ERISA and its Form 5500

3.2.11 HIPAA and regulatory compliance

3.2.12 Pharmacy benefit management (PBM)

3.2.13 COBRA management

3.2.14 Disease management

3.2.15 Utilization review

3.2.16 Wellness programs
3.2.17 Customer Service
3.2.18 Fulfillment

3.3 Authority of Plan Administrator: The Plan Administrator has the sole and absolute discretion and authority to:

3.3.1 interpret the terms of the Plan,

3.3.2 to determine factual questions that arise in the course of administering the Plan,

3.3.3 to adopt rules and regulations regarding the administration of the Plan, including enrollment procedures,

3.3.4 In collaboration with the Company, construct and devise the benefit structure and contributions to the Plan in order to, in its opinion, ensure the viability of the Plan.

3.3.5 to determine the conditions under which benefits become payable under the Plan,

3.3.6 to make determinations of eligibility under the Plan,

3.3.7 to verify the initial and continuing eligibility for participation and benefits under the Plan of any person, including any child, spouse, domestic partner, or dependent of an employee, by requesting proof of such eligibility including, as applicable and without limitation, tax returns, marriage certificates, birth certificates, proof of residence, proof of domestic partnership, or other documentation deemed appropriate by the Plan Administrator, and to make any other determinations that the Plan Administrator believes are necessary and advisable for the administration of the Plan.

3.3.8 Any interpretation or determination made by the Plan Administrator will be final, conclusive, and binding on all parties, however, under no circumstances can the Plan be modified by oral statements of the Plan Administrator.

3.4 Indemnification of Plan Administrator:

3.4.1 Each of the parties acknowledge that by entering into and performing its obligations under this Agreement, Plan Administrator

will not assume and should not be exposed to the business and operational risks associated with Companies business, and Company therefore agrees to indemnify and defend Plan Administrator from any and all claims, actions, damages, liabilities, costs and expenses, including reasonable attorneys' fees and expenses (collectively, "Losses") arising out of, under or in connection with any claim relating to the conduct of Companies business, including the Company's obligations under the Plan.

3.4.2 The Company agrees to indemnify and to defend to the fullest extent permitted by law the Plan Administrator against all liabilities, damages, costs, and expenses (including attorney's fees and amounts paid in settlement of any claims approved by the Company) occasioned by any act or omission to act in connection with the Plan, if such act or omission is in good faith.

3.5 Compensation of Plan Administrator: As per contract

3.6 Fiduciary Statement: The Plan Administrator is a "named fiduciary" with respect to the Plan. A named fiduciary can appoint others to carry out fiduciary responsibilities (other than as a trustee) under the Plan. These other persons become fiduciaries themselves and are responsible for their acts under the Plan. To the extent that the named fiduciary allocates its responsibility to other persons, the named fiduciary will not be liable for any act or omission of such person unless either (i) the named fiduciary has violated its stated duties under ERISA in appointing the fiduciary, establishing the procedures to appoint the fiduciary, or continuing either the appointment or the procedures, or (ii) the named fiduciary has breached its fiduciary responsibility under section 405(a) of ERISA.

3.7 Access to Documents: The Plan Administrator will generally make available to each Eligible Employee such of his or her records under the Plan as pertain to him or her for examination at reasonable times during normal business hours, but the Plan Administrator shall have no obligation to disclose any records or information that the Plan Administrator, in its sole discretion, determines to be of a privileged or confidential nature.

3.8 Contact Information: MBA 18700 N Hayden Rd Suite 390, Scottsdale, AZ 85255. 480-576-7118.

SEC 4: ELIGIBILITY AND ENROLLMENT

4.1 Eligibility: If you are an eligible W2 employee, 1099 affiliate or working owner of the Company s and other enrollment requirements as stated below, you can enroll for coverage. In addition, certain members of your immediate family may also be eligible, so long as the Eligible Employee is enrolled in the Plan, and they meet the eligible dependent definitions. Upon request, Eligible Employees must provide proof of their dependent's eligibility for coverage.

4.2 Enrollment: In order to enroll in the Plan, you must complete the enrollment forms and process prescribed by the Plan Administrator by a prescribed deadline prior to commencement of coverage under the Plan. As a requirement of enrollment in the Plan, the Plan Administrator may require that you and your Eligible Dependents provide certain personal information including, without limitation, addresses and Social Security numbers.

4.3 Timing of enrollment and enrollment changes: Your opportunities to enroll in the Plan, as well as to change or cancel your enrollment in the Plan, are limited to the following:

4.3.1 You (and your Eligible Dependents) may enroll at any time;

4.3.2 You (and your Eligible Dependents) change or cancel your enrollment, at any time; or

4.3.3 you or your Eligible Dependent initially declined coverage because you had other health care coverage and you (or your Eligible Dependent) have lost eligibility for that other health care coverage through no fault of your (or his or her) own, or

4.3.4 since declining coverage initially, you have acquired a new dependent (through marriage or the birth or adoption or placement for adoption of a child) and wish to cover that person.

4.3.5 In the former case, you must have given (in writing if a written statement was required at the time by the Plan Administrator and you were provided with a notice of that requirement and its consequences at that time) the alternative coverage as your reason for waiving coverage under the Plan when you declined to participate.

4.3.6 In either case, as long as you (and/or your Eligible Dependents) meet the necessary requirements under the Plan (including eligibility requirements), you can enroll both yourself and all Eligible Dependents in the Plan within 30 days after you lose your alternative coverage or the date of your marriage or the birth, adoption, or placement for adoption of your child.

4.3.7 Enrollment of your Eligible Dependents is generally conditioned upon your enrollment.

4.3.8 You may also be able to enroll yourself and your dependent in the Plan pursuant to a special enrollment right created by the Children's Health Insurance Program Reauthorization Act of 2009. If you or your

dependent is eligible for, but not enrolled, for coverage under the terms of the Plan, you (and/or your dependent) may enroll for coverage under the terms of the Plan if either of the following conditions are met:

4.3.8.1 You or your dependent is covered under a Medicaid plan under title XIX of the Social Security Act or under a State child health plan under Title XXI of such Act and your (or your dependent's) coverage under such plan is terminated as a result of loss of eligibility for such coverage and you request coverage under the Plan not later than 60 days after the termination of such coverage; or

4.3.8.2 You or your dependent become eligible for assistance, with respect to coverage under the Plan, under such Medicaid plan or state child health plan (including under any waiver or demonstration project conducted under or in relation to such a plan), if you request coverage under the Plan not later than 60 days after the date you or your dependent is determined to be eligible for such assistance.

4.3.9 Please contact the Plan Administrator for details about special enrollment.

4.4 Qualified Medical Child Support Orders: A Qualified Medical Child Support Order, as defined under Section 609 of ERISA ("QMCSO") is an order by a court for one parent to provide a child or children with health insurance under the Plan. The Plan Administrator will comply with the terms of any QMCSO it receives, and will:

4.4.1 Establish reasonable procedures to determine whether medical child support orders are QMCSOs;

4.4.2 Promptly notify you and any alternate recipient (as defined in Section 609(a)(2)(C) of ERISA) of the receipt of any medical child support order, and Plan's procedures for determining whether medical child support orders are QMCSOs; and

4.4.3 Within a reasonable period of time after receipt of such order, the Plan Administrator will determine whether such order is a QMCSO and will notify you and each alternate recipient of such determination.

4.4.4 A copy of the Plan's QMCSO procedures is available, without charge, upon request from the Plan Administrator.

4.5 When Coverage Ends: Except in the case of COBRA rights described below, benefits for you and your Eligible Dependents under the Plan will terminate upon the earliest of:

4.5.1 The date you cease to be an Eligible Employee,

4.5.2 For your Eligible Dependents, the date your Eligible Dependent cease to be your Eligible Dependents;

4.5.3 The time when you or your Eligible Dependent(s) have exhausted the benefits available under the Plan,

4.5.4 The last day for which necessary contributions are made to the Plan for your or your Eligible Dependents' coverage,

4.5.5 The date when the Company amends the Plan to eliminate eligibility for you and/or your Eligible Dependents, or

4.5.6 The date the Company terminates the Plan.

4.5.7 As noted below, the Company reserves the right to change or eliminate benefits under the Plan and may amend or terminate the Plan at any time.

SEC 5: CONTRIBUTIONS

5.1 Amounts: The Plan provides one on more benefits levels as more fully described in the Appendix 1. The Plan also provides levels of benefit coverage, employee only, employee and spouse, employee and child(ren) and family. The monthly amount you contribute to the cost of your benefits will vary depending on the level of benefits and level of coverage you select when you enroll. The Company and Plan Administrator reserve

the right to change the benefits and costs of the plan, at any time, in accordance with the provisions of ERISA and/or other relevant statutes or regulations.

5.2 The Plan Administrator may require that your contributions be made by payroll deduction. In this case, your contributions will be used in funding the cost of the Plan benefits as soon as practicable after they have been received from you or withheld from your pay through payroll deduction.

SEC 6: CLAIMS AND INTERNAL APPEAL PROCEDURE

The procedure(s) for submitting a claim and for appealing an adverse coverage decision are below, PROVIDED HOWEVER that at all times the procedure(s) provided for benefits required by the ACA shall be employed by the Plan Administrator. A Participant may initiate a claim for benefits under the Plan by contacting the Plan Administrator. The Plan Administrator will inform the Participant of the information needed to complete the claim, which may include:

6.1 A completed claim form, in a form provided by the Plan Administrator;

6.2 Reasonable documentation from the Participant's physician or other provider or official describing and/or verifying the injury, illness, or other condition or event giving rise to the Participant's claim;

6.3 Copies of bills for services rendered, including the Participant's name; the name, address, and telephone number of the provider; the diagnosis; the type of services rendered, with diagnosis and/or procedure codes; the date of services; and the charges.

6.4 Claims should be filed with the Plan Administrator within 60 days of the date charges for the services were incurred. Benefits are based on the Plan's provisions at the time the charges were incurred. Claims filed later than that date may be declined or reduced unless:

6.4.1 It is not reasonably possible to submit the claim in that time; and

6.4.2 The claim is submitted within one year from the date incurred.

6.5 The Plan Administrator will determine if enough information has been submitted to enable proper consideration of the claim. If not, more information may be requested from the Participant. The Plan reserves the right to have a Participant seek a second medical opinion.

6.6 A request for Plan benefits will be considered a claim for Plan benefits, and it will be subject to a full and fair review.

6.7 Timing of Initial Decision: The period of time within which a benefit determination is required to be made shall begin at the time a claim is filed in accordance with the procedures set forth above, without regard to whether all the information necessary to make a benefit determination accompanies the filing. In the event that a period of time is extended due to a Participant's failure to submit information necessary to decide a claim, the period for making the benefit determination shall be tolled from the date on which the notification of the extension is sent to the Participant until the date on which the Participant responds to the request for additional information.

6.8 Post-Service Claims: Post-Service Claims are those claims that are filed for payment of benefits after health care has been received.

6.8.1 If your Post-Service Claim is denied, you will receive a written notice of the claim decision (whether or not adverse) from the Plan Administrator within a reasonable period of time, but not later than 30 days, following the receipt of the claim, as long as all needed information was provided with the claim.

6.8.2 The Plan Administrator will notify you of its determination within 30 days after the claim is received, unless the Plan Administrator determines, in its discretion, that special circumstances require an extension of time for processing the claim.

6.8.3 If an extension of time is required, a written or electronic extension notice indicating the special circumstances requiring the extension of time and the date by which the Plan Administrator expects to render a decision shall be furnished to you prior to the end of the initial 30-day period.

6.8.4 If the extension is necessary because of your failure to provide missing information and you are notified of that fact, the extension shall not exceed a period of 15 days beginning as of the earlier of (1) the date the missing information is received by the Plan Administrator or (2) the end of a 45-day period afforded to you to provide the missing information.

6.8.5 If all of the needed information is received within the 45-day time frame, the Plan Administrator will notify you of the determination within 15 days after the information is received.

6.8.6 If you do not provide the needed information within the 45-day period, your claim will be denied.

6.8.7 If the extension is requested for reasons other than your failure to provide missing information, the extension shall not exceed 15 days from the end of the initial 30-day period.

6.9 Pre-Service Claims: are those claims that require notification or approval prior to receiving health care. If your claim was a Pre-Service Claim, and was submitted properly with all needed information, you will receive written notice of the claim decision (whether or not adverse) from the Plan Administrator within a reasonable period of time, but not later than 15 days, following the receipt of the claim.

6.9.1 If you filed a Pre-Service Claim improperly, the Plan Administrator will notify you of the improper filing and how to correct it within 5 days of receipt of the Pre-Service Claim.

6.9.2 The Plan Administrator will notify you of its determination within 15 days after the claim is received, unless the Plan administrator determines, in its discretion, that special circumstances require an extension of time for processing the claim.

6.9.3 If an extension of time is required, a written or electronic extension notice indicating the special circumstances requiring the extension of time and the date by which the Plan Administrator expects to render a decision shall be furnished to you prior to the end of the initial 15-day period.

6.9.4 If the extension is necessary because of your failure to provide missing information and you are notified of that fact, the extension shall not exceed a period of 15 days beginning as of the earlier of (1) the date the missing information is received by the Plan Administrator or (2) the end of a 45-day period afforded to you to provide the missing information.

6.9.5 If all of the needed information is received within the 45-day time frame, the Plan Administrator will notify you of the determination within 15 days after the information is received. If you do not provide the needed information within the 45-day period, your claim will be denied.

6.9.6 If the extension is requested for reasons other than your failure to provide missing information, the extension shall not exceed 15 days from the end of the initial 15-day period.

6.10 Urgent Claims That Require Immediate Action: means are those claims that require notification or approval prior to receiving medical care, where a delay in treatment could seriously jeopardize your life or health or the ability to regain maximum function or, in the opinion of a doctor with knowledge of your health condition, could cause severe pain. In these situations:

6.10.1 You will receive notice of the benefit determination (whether or not adverse) in writing or electronically as soon as possible taking into account the medical exigencies, but not later than 72 hours after the Plan Administrator receives all necessary information.

6.10.2 Notice of denial may be oral with a written confirmation to follow within 3 days.

6.10.3 If you filed an Urgent Care Claim improperly, the Plan Administrator will notify you of the improper filing and how to correct it within 24 hours after the Urgent Care Claim was received. If additional information is needed to process the claim, the Plan Administrator will notify you of the information needed within 24 hours after the claim was received. You then have 48 hours to provide the requested information.

6.10.4 In determining whether a claim is urgent, the Plan Administrator shall defer to the determination of a Participant's attending provider.

6.10.5 You will be notified of a determination no later than 48 hours after:

6.10.5.1 The Plan Administrator's receipt of the requested information; or

6.10.5.2 The end of the 48-hour period within which you were to provide the additional information, if the information is not received within that time.

6.10.6 Special Rules for Concurrent Decisions: Participant's request to extend previously approved course of treatment.

6.10.6.1 Urgent care. If an ongoing course of treatment was previously approved for a specific period of time or number of treatments, and a request to extend the treatment is an Urgent Care Claim, the request will be decided by the Plan Administrator within 24 hours of the receipt of the request, provided the request is made at least 24 hours prior to the end of the approved treatment. If the request for extended treatment is not made at least 24 hours prior to the end of the approved treatment, the request will be treated as an Urgent Care Claim and decided according to the time frames described above.

6.10.6.2 Non-Urgent care. If an ongoing course of treatment was previously approved for a specific period of time or number of treatments, and a request to extend treatment is not an Urgent Care Claim, the request will be considered a new claim and decided according to the post-service or pre-service time frames described above, whichever applies.

6.10.7 Plan reduces or terminates a previously approved course of treatment. If an ongoing course of treatment was previously approved for a specific period of time or number of treatments, and the Plan reduces or terminates such course of treatment

(other than by Plan amendment or termination) before the end of such period of time or number of treatments, the reduction or termination shall be considered an Adverse Benefit Determination (as defined below) and you shall be notified of the reduction or termination (sufficiently in advance of the termination or reduction to appeal the decision and obtain a determination upon review of the decision) before the course of treatment is reduced or terminated.

6.11 Notice of Initial Internal Adverse Benefit Determination: If your claim is wholly or partially denied, or if there occurs a rescission of coverage (within the meaning of Public Health Service Act Section 2712 and regulations issued thereunder) under the Plan (each, an Adverse Benefit Determination and the initial claim denial, an "Initial Internal Adverse Benefit Determination") the Plan Administrator will furnish the Participant with a written notice of the Adverse Benefit Determination. The written notice will set forth the following information, in a manner calculated to be understood by the Participant:

6.11.1 The specific reason or reasons for the Initial Internal Adverse Benefit Determination.

6.11.2 Specific reference to those Plan provisions on which the Initial Internal Adverse Benefit Determination is based.

6.11.3 A description of any additional information or material necessary to perfect the claim and an explanation of why such material or information is necessary.

6.11.4 Appropriate information as to the steps to be taken if you wish to submit the claim for review.

6.11.5 If an internal rule, guideline, protocol, or other similar criterion was relied upon in making the Initial Internal Adverse Benefit Determination, either the specific rule, guideline, protocol, or other similar criterion; or a statement that such a rule, guideline, protocol, or other similar criterion was relied upon in making the Adverse Benefit Determination and that a copy of such rule,

guideline, protocol, or other criterion will be provided free of charge to the Participant upon request.

6.11.6 If the Initial Internal Adverse Benefit Determination is based on a medical necessity or experimental treatment or similar exclusion or limit, either an explanation of the scientific or clinical judgment for the determination, applying the terms of the Plan to the Participant's medical circumstances, or a statement that such explanation will be provided free of charge upon request.

6.11.7 In the case of an Initial Internal Adverse Benefit Determination concerning a claim involving urgent care, a description of the expedited review process applicable to such claims.

6.11.8 A statement indicating that the Participant shall be provided, upon request and free of charge, with reasonable access to, and copies of, all documents, records, and other information relevant to the Participant's claim for benefits.

6.11.9 The Plan will ensure that any notice of Initial Internal Adverse Benefit Determination includes information sufficient to identify the claim involved (including the date of service, the health care provider, and the claim amount (if applicable).

6.11.10 The Participant shall be provided, upon request and free of charge, the diagnosis code and its corresponding meaning, and the treatment code and its corresponding meaning, and the notice shall contain a statement to such effect. The Plan will not consider any such request, in itself, to be a request for internal appeal or External Review.

6.11.11 The Plan must ensure that the reason or reasons for the Initial Internal Adverse Benefit Determination includes the denial code and its corresponding meaning, as well as a description of the Plan's standard, if any, that was used in denying the claim.

6.11.12 The Plan must provide a description of available internal appeals and External Review processes, including information regarding how to initiate an appeal.

6.11.13 The Plan must disclose the availability of, and contact information for, any applicable office of health insurance consumer assistance or ombudsman established under Section 2793 of the Public Health Service Act to assist individuals with the internal claims and appeals and External Review processes.

6.12 Appeals of Initial Internal Adverse Benefit Determination: If you disagree with a claim determination after following the above steps, you can contact the Plan Administrator in writing to formally request an appeal. In your appeal, you may submit written comments, documents, records, and other information relating to your claim for benefits. You shall be provided, upon request and free of charge, reasonable access to, and copies of, all documents, records, and other information relevant to your claim for benefits. The review of your claims shall take into account all comments, documents, records, and other information you submit, without regard to whether such information was submitted or considered in the initial benefit determination. The Plan will identify, upon request to the Plan Administrator, any medical or vocational experts whose advice was obtained on behalf of the Plan in connection with your Initial Internal Adverse Benefit Determination, without regard to whether the advice was relied upon in making the benefit determination. If the appeal relates to a claim for payment, your request should include:

6.12.1 The patient's name and the identification number from the ID card.

6.12.2 The date(s) of health care service(s).

6.12.3 The provider's name.

6.12.4 The reason you believe the claim should be paid.

6.12.5 Any documentation or other written information to support your request for claim payment.

6.12.6 You may appeal any denial of a claim within 180 days of receipt of such a denial by submitting a written request for review to the Plan Administrator.

6.12.7 The review of your appeal shall not afford deference to the Initial Internal Adverse Benefit Determination and shall be conducted by an appropriate named fiduciary of the Plan who is neither the individual who made the Initial Internal Adverse Benefit Determination that is the subject of the appeal, nor the subordinate of such individual. In deciding an appeal that is based in whole or in part on a medical judgment, including determinations with regard to whether a particular treatment, drug, or other item is experimental, investigational, or not medically necessary or appropriate, the appropriate named fiduciary shall consult with a health care professional who has appropriate training and experience in the field of medicine involved in the medical judgment and who is neither an individual who was consulted in connection with the Initial Internal Adverse Benefit Determination that is the subject of the appeal, nor the subordinate of any such individual.

6.12.8 In the case of a claim under the Plan involving urgent care, you are entitled to an expedited review process pursuant to which:

6.12.8.1 You may submit a request for an expedited appeal of an Initial Internal Adverse Benefit Determination orally or in writing: and

6.12.8.2 All necessary information, including the Plan's benefit determination on review, shall be transmitted from the Plan to you by telephone, facsimile, or other available similarly expeditious method.

6.13 Timing of Notification of Benefit Determination on Review: For purposes of this section, the period of time within which a benefit determination on review is required to be made shall begin at the time an appeal is filed without regard to whether all the information necessary to make a benefit determination on review accompanies the filing. In the event that

a period of time is extended as permitted below due to a Participant's failure to submit information necessary to decide a claim, the period for making the benefit determination on review shall be tolled from the date on which the notification of the extension is sent to the Participant until the date on which the Participant responds to the request for additional information.

6.14 Appeal Process: A qualified individual who was not involved in the decision being appealed will be appointed to decide the appeal. If your appeal is related to clinical matters, the review will be done in consultation with a health care professional with appropriate expertise in the field who was not involved in the prior determination. The Plan Administrator may consult with, or seek the participation of, medical experts as part of the appeal resolution process. You hereby consent to this referral and the sharing of pertinent health claim information. Upon request and free of charge you have the right to reasonable access to, and copies of, all documents, records, and other information relevant to your claim for benefits.

6.15 Appeals Determinations: Pre-Service and Post-Service Claim Appeals, you will be provided with written or electronic notification of the decision on your appeal as follows:

6.15.1 For appeals of Pre-Service Claims (as defined previously), a first level appeal will be conducted, and you will be notified by the Plan Administrator of the decision within a reasonable period of time appropriate to the medical circumstances, but not later than 15 days from receipt of a request for appeal of a denied claim. If you are not satisfied with the first level appeal decision of the Plan Administrator, you have the right to request a second level appeal from the Plan Administrator. Your second level appeal request must be submitted to the Plan Administrator within 60 days of the receipt of the first level appeal decision. The second level appeal will be conducted, and you will be notified by the Plan Administrator of the decision within a reasonable period of time appropriate to the medical circumstances, but not later than

15 days from receipt of a request for review of the first level appeal decision.

6.15.2 For appeals of Post-Service Claims (as defined previously), the first level appeal will be conducted, and you will be notified by the Plan Administrator of the decision within a reasonable period of time appropriate to the medical circumstances, but not later than 30 days from receipt of a request for appeal of a denied claim. If you are not satisfied with the first level appeal decision of the Plan Administrator, you have the right to request a second level appeal from the Plan Administrator. Your second level appeal request must be submitted to the Plan Administrator within 60 days of the receipt of the first level appeal decision. The second level appeal will be conducted, and you will be notified by the Plan Administrator of the decision within a reasonable period of time appropriate to the medical circumstances, but not later than 30 days from receipt of a request for review of the first level appeal decision.

6.15.3 For procedures associated with Urgent Claims, see "Urgent Claim Appeals That Require Immediate Action" below.

6.15.4 Please note that the Plan Administrator's decision is based only on whether or not benefits are available under the Plan for the proposed treatment or procedure. The determination as to whether the pending health service is right for you is between you and your doctor.

6.16 Urgent Claim Appeals That Require Immediate Action: Your appeal may require immediate action if a delay in treatment could significantly increase the risk to your health or the ability to regain maximum function or cause severe pain. In these urgent situations:

6.16.1 The appeal does not need to be submitted in writing. You or your doctor should call the Plan Administrator as soon as possible. The Plan Administrator will provide you with a written or electronic determination within 72 hours following receipt of your request for

review of the determination taking into account the seriousness of your condition.

6.16.2 If the Initial Internal Adverse Benefit Determination involves a medical condition for which the time frame for completion of an internal appeal would seriously jeopardize your life or health or would jeopardize your ability to regain maximum function, you may file a request with the Plan Administrator for an "expedited external review" within the meaning of regulations issued under section 2719 of the Public Health Service Act.

6.16.3 The Plan Administrator has the exclusive right to interpret and administer the provisions of the Plan. The Plan Administrator's decisions are conclusive and binding. The Plan Administrator has final claims adjudication authority under the Plan.

6.17 Manner of Notification of Final Internal Benefit Determination: The Plan Administrator shall provide a Participant with written or electronic notification of a Plan's benefit determination on review. In the case of an Adverse Benefit Determination (a "Final Internal Adverse Benefit Determination"), the notification shall set forth, in a manner calculated to be understood by the Participant:

6.17.1 The specific reason or reasons for the Final Internal Adverse Benefit Determination.

6.17.2 Reference to the specific Plan provisions on which the Final Internal Adverse Benefit Determination is based.

6.17.3 A statement that the Participant is entitled to receive, upon request and free of charge, reasonable access to, and copies of, all documents, records, and other information relevant to the Participant's claim for benefits.

6.17.4 A statement describing any voluntary appeal procedures offered by the Plan and the Participant's right to obtain the information about such procedures.

6.17.5 A statement of the Participant's right to bring an action under section 502(a) of ERISA.

6.17.6 If an internal rule, guideline, protocol, or other similar criterion was relied upon in making the Final Internal Adverse Benefit Determination, either the specific rule, guideline, protocol, or other similar criterion; or a statement that such rule, guideline, protocol, or other similar criterion was relied upon in making the Adverse Benefit Determination and that a copy of the rule, guideline, protocol, or other similar criterion will be provided free of charge to the Participant upon request.

6.17.7 If the Final Internal Adverse Benefit Determination is based on a medical necessity or experimental treatment or similar exclusion or limit, either an explanation of the scientific or clinical judgment for the determination, applying the terms of the Plan to the Participant's medical circumstances, or a statement that such explanation will be provided free of charge upon request.

6.17.8 The following statement: "You and your plan may have other voluntary alternative dispute resolution options, such as mediation. One way to find out what may be available is to contact your local US Department of Labor Office and your state insurance regulatory agency."

6.17.9 The Plan will ensure that any notice of Initial Internal Adverse Benefit Determination includes information sufficient to identify the claim involved (including the date of service, the health care provider, and the claim amount (if applicable).

6.17.10 The Participant shall be provided, upon request and free of charge, the diagnosis code and its corresponding meaning, and the treatment code and its corresponding meaning, and the notice shall contain a statement to such effect. The Plan will not consider any such request, in itself, to be a request for External Review.

6.17.11 The Plan will ensure that the reason or reasons for the Initial Internal Adverse Benefit Determination includes the denial code and its corresponding meaning, as well as a description of the Plan's standard, if any, that was used in denying the claim. The description will also include a discussion of the decision.

6.17.12 The Plan will provide a description of available internal appeals and External Review processes, including information regarding how to initiate an appeal.

6.17.13 The Plan will disclose the availability of, and contact information for, any applicable office of health insurance consumer assistance or ombudsman established under Section 2793 of the Public Health Service Act to assist individuals with the internal claims and appeals and External Review processes.

6.18 Voluntary Extensions: As described above, the Plan Administrator must decide your claim and/or appeal within certain time frames, and the Plan Administrator may extend those time frames in its discretion in certain circumstances. In addition, the Plan Administrator may request that you voluntarily agree to allow the Plan Administrator additional time extensions. You may allow or deny these additional "voluntary' extensions in your discretion.

6.19 Rules Generally Applicable to Claims under the Plan: The following appeals procedure applies to claims made for benefits under the Plan:

6.19.1 The Company must allow you to review the claim file and to present evidence as part of the internal claims and appeals process.

6.19.2 Any decision regarding hiring, compensation, termination, promotion, or similar matters with respect to an individual such as a claims adjudicator or a medical expert must not be based upon the likelihood that the individual will support a denial of benefits.

6.19.3 The Plan must provide you, free of charge, with any new or additional evidence considered, relied upon, or generated by the Plan (or at the direction of the Plan) in connection with your claim; such evidence must be provided as soon as possible and sufficiently in advance of the date on which the notice of Final Internal Adverse Benefit Determination is required to be provided (see Timing of Notification of Benefit Determination on Review) to give you a reasonable opportunity to respond prior to that date.

6.19.4 Before the Plan can issue a Final Internal Adverse Benefit Determination based on a new or additional rationale, you must be provided, free of charge, with the rationale; the rationale must be provided as soon as possible and sufficiently in advance of the date on which the notice of Final Internal Adverse Benefit Determination is required to be provided (see Timing of Notification of Benefit Determination on Review, above) to give you a reasonable opportunity to respond prior to that date. If the new or additional evidence is received so late that it would be impossible to provide it to the claimant in time for the claimant to have a reasonable opportunity to respond, the period for providing a Final Internal Adverse Benefit Determination is tolled until such time as the claimant has a reasonable opportunity to respond. After the claimant responds or has a reasonable opportunity to respond but fails to do so, the Plan Administrator shall notify the claimant of the Plan's benefit determination as soon as a Plan acting in a reasonable and prompt fashion can provide the notice, taking into account the medical exigencies.

6.20 Authorized Representatives: Any reference in these procedures to "you" or the "Participant" is also a reference to the Participant's or your authorized representative making a claim on his or her behalf. The Plan Administrator reserves the right to establish reasonable procedures for determining whether an individual has been authorized to act on your and/or the Participant's behalf.

6.21 Questions About Your Claims and Appeal Rights: For questions about your rights, these claims procedures, or for assistance, you can contact the Employee Benefits Security Administration at 1.866.444.EBSA (3272).

SEC 7: CLAIMS AND EXTERNAL APPEAL PROCEDURE

Under these procedures: you may be entitled to request an External Review of a Final Internal Adverse Benefit Determination by the Plan (an "External Review"); and if your situation is urgent, you may be entitled to an Expedited External Review of an Adverse Benefit Determination by the Plan (an "Expedited External Review").

For purposes of these External Review Procedures, an "Adverse Benefit Determination" means an adverse benefit determination as defined in 29 CFR 2560.503-1 that involves medical judgment, as well as any rescission of coverage, as described in Public Health Service Act Section 2712 (whether or not, in connection with the rescission, there is an adverse effect on any particular benefit at that time), and may be either an initial internal Adverse Benefit Determination ("Initial Internal Adverse Benefit Determination") or an internal Adverse Benefit Determination made on appeal thereof (a "Final Internal Adverse Benefit Determination").

Any reference in these procedures to "you" or the "Participant" is also a reference to the Participant's or your authorized representative making a claim on his, her or your behalf. The Plan Administrator reserves the right to establish reasonable procedures for determining whether an individual has been authorized to act on your and/or the Participant's behalf.

7.1 Standard External Review: Standard External Review is External Review that is not considered expedited (as described below).

7.1.1 Request for External Review. The Plan Administrator will allow you to file a request for an External Review with the Plan Administrator if the request is filed within four months after the date of receipt of a notice of an Adverse Benefit Determination. If there is no corresponding date four months after the date of receipt of such a notice, then the request must be filed by the first day of the fifth month following the receipt of the notice. For example, if the date of receipt of the notice is October 30, because there is no February 30, the request must be filed by March 1. If the last filing date

would fall on a Saturday, Sunday, or federal holiday, the last filing date is extended to the next day that is not a Saturday, Sunday, or federal holiday.

7.1.2 Preliminary review. Within five business days following the date of receipt of the External Review request, the Plan Administrator will complete a preliminary review of the request to determine whether you meet all of the following requirements for Standard External Review:

7.1.3 You are or were covered under the Plan at the time the health care item or service was requested or, in the case of a retrospective review, were covered under the Plan at the time the health care item or service was provided;

7.1.4 The Adverse Benefit Determination does not relate to the your failure to meet the requirements for eligibility under the terms of the Plan (e.g., worker classification or similar determination);

7.1.5 You have exhausted the Plan's internal appeal process unless you are not required to exhaust the internal appeals process under applicable regulations; and

7.1.6 You have provided all the information and forms required to process an External Review.

7.1.7 Within one business day after completion of the preliminary review, the Plan Administrator will issue to you a notification in writing. If the request is complete but not eligible for External Review, such notification must include the reasons for its ineligibility and contact information for the Employee Benefits Security Administration (toll-free number 1.866.444.EBSA [3272]). If the request is not complete, such notification must describe the information or materials needed to make the request complete and the Company will allow a Participant to perfect the request for External Review within the four-month filing period or within the 48-hour period following the receipt of the notification, whichever is later.

7.1.8 Referral to IRO. The Plan Administrator will assign an independent review organization ("IRO") that is accredited by URAC or a similar nationally recognized accrediting organization to conduct the External Review. Moreover, the Plan Administrator will take action against bias and to ensure independence. Accordingly, the Plan Administrator will contract with IROs for assignments under the Plan and rotate claims assignments among them (or incorporate other independent, unbiased methods for selection of IROs, such as random selection. The Plan Administrator will contract with at least three IROs. In addition, the IRO may not be eligible for any financial incentives based on the likelihood that the IRO will support the denial of benefits. The IRS may not impose any costs, including filing fees, on the individual requesting External Review.

7.1.9 A contract between the Plan and an IRO will provide the following:

7.1.9.1 The assigned IRO will utilize legal experts where appropriate to make coverage determinations under the Plan.

7.1.9.2 The assigned IRO will timely notify the Participant in writing of the request's eligibility and acceptance for External Review. This notice will include a statement that the Participant may submit in writing to the assigned IRO within 10 business days following the date of receipt of the notice additional information that the IRO must consider when conducting the External Review. The IRO is not required to, but may, accept and consider additional information submitted after 10 business days.

7.1.9.3 Within five business days after the date of assignment of the IRO, the Plan Administrator will provide to the assigned IRO the documents and any information considered in making the adverse benefit determination or final internal adverse benefit determination. Failure by the Plan Administrator to timely provide the documents and information must not

delay the conduct of the External Review. If the Plan Administrator fails to timely provide the documents and information, the assigned IRO may terminate the External Review and make a decision to reverse the Adverse Benefit Determination. Within one business day after making the decision, the IRO must notify the Participant and the Plan Administrator.

7.1.10 Upon receipt of any information submitted by the Participant, the assigned IRO will within one business day forward the information to the Plan Administrator. Upon receipt of any such information, the Plan Administrator may reconsider the Adverse Benefit Determination that is the subject of the External Review. Reconsideration by the Plan Administrator must not delay the External Review. The External Review may be terminated as a result of the reconsideration only if the Plan Administrator decides, upon completion of its reconsideration, to reverse its Adverse Benefit Determination and provide coverage or payment. Within one business day after making such a decision, the Plan Administrator must provide written notice of its decision to the Participant and the assigned IRO. The assigned IRO must terminate the External Review upon receipt of the notice from the Plan Administrator.

7.1.11 The IRO will review all of the information and documents timely received. In reaching a decision, the assigned IRO will review the claim de novo and not be bound by any decisions or conclusions reached during the Plan's internal claims and appeals process applicable under paragraph (b) of the final regulations issued under section 2719 of the Public Health Service Act. In addition to the documents and information provided, the assigned IRO, to the extent the information or documents are available, and the IRO considers them appropriate, will consider the following in reaching a decision:

7.1.11.1 The Participant's medical records;

7.1.11.2 The attending health care professional's recommendation;

7.1.11.3 Reports from appropriate health care professionals and other documents submitted by the Company or issuer, Participant, or the Participant's treating provider;

7.1.11.4 The terms of the Plan to ensure that the IRO's decision is not contrary to the terms of the Plan, unless the terms are inconsistent with applicable law;

7.1.11.5 Appropriate practice guidelines, which must include applicable evidence-based standards and may include any other practice guidelines developed by the federal government, national or professional medical societies, boards, and associations;

7.1.11.6 Any applicable clinical review criteria developed and used by the Plan or the Plan Administrator, unless the criteria are inconsistent with the terms of the Plan or with applicable law; and

7.1.11.7 To the extent the final IRO decision maker is different than the IRO's clinical reviewer, the opinion of such clinical reviewer or reviewers after considering the information described in this notice to the extent the information or documents are available and the clinical reviewer or reviewers consider appropriate.

7.1.11.8 The assigned IRO must provide written notice of the final External Review decision within 45 days after the IRO receives the request for the External Review. The IRO must deliver the notice of final External Review decision to the claimant and the Plan. The assigned IRO's decision notice will contain:

7.1.11.9 A general description of the reason for the request for External Review, including information sufficient to identify the claim (including the date or dates of service, the health care provider, the claim amount (if applicable),

and a statement describing the availability upon request of the diagnosis code and its corresponding meaning,

7.1.11.10 The treatment code and its corresponding meaning, and the reason for the Plan's previous denial;

7.1.11.11 The date the IRO received the assignment to conduct the External Review and the date of the IRO decision;

7.1.11.12 References to the evidence or documentation, including the specific coverage provisions and evidence-based standards, considered in reaching its decision;

7.1.11.13 A discussion of the principal reason or reasons for its decision, including the rationale for its decision and any evidence-based standards that were relied on in making its decision;

7.1.11.14 A statement that the determination is binding except to the extent that other remedies may be available under state or federal law to either the Plan or to the Participant, or to the extent the Plan voluntarily makes payment on the claim or otherwise provides benefits at any time, including after a final External Review decision that denies the claim or otherwise fails to require such payment or benefits;

7.1.11.15 A statement that judicial review may be available to the Participant; and

7.1.11.16 Current contact information, including phone number, for any applicable office of health insurance consumer assistance or ombudsman established under Public Health Service Act Section 2793.

7.1.12 After a final External Review decision, the IRO must maintain records of all claims and notices associated with the External Review process for six years. An IRO must make such records available for examination by the Participant, Plan, or state or federal oversight agency upon request, except where such disclosure would violate state or federal privacy laws.

7.1.13 Reversal of Plan's decision. Upon receipt of a notice of a final External Review decision reversing the Adverse Benefit Determination, the Plan immediately must provide coverage or payment (including immediately authorizing or immediately paying benefits) for the claim.

7.2 Expedited External Review:

7.2.1 The Plan must allow you to make a request for an expedited External Review with the Plan at the time you receive:

7.2.1.1 An Initial Internal Adverse Benefit Determination that involves a medical condition of yours for which the time frame for completion of an expedited internal appeal under the interim final regulations under Section 2719 of the Public Health Service Act would seriously jeopardize your life or health or would jeopardize your ability to regain maximum function and you have filed a request for an expedited internal appeal; or

7.2.1.2 Final Internal Adverse Benefit Determination, if you have a medical condition where the time frame for completion of a Standard External Review would seriously jeopardize your life or health or would jeopardize your ability to regain maximum function, or if the Final Internal Adverse Benefit Determination concerns an admission, availability of care, continued stay, or health care item or service for which you received emergency services, but have not been discharged from a facility.

7.2.2 Preliminary review. Immediately upon receipt of the request for expedited External Review, the Plan Administrator will determine whether the request meets the reviewability requirements set forth in paragraph A.2 above for Standard External Review. The Plan Administrator will immediately send a notice that meets the requirements set forth in paragraph A.2 above for Standard External Review to the Participant of its eligibility determination.

7.2.3 Referral to IRO. Upon a determination that a request is eligible for External Review following the preliminary review, the Company will assign an IRO pursuant to the requirements set forth in paragraph xx above for Standard External Review. The Plan Administrator will provide or transmit all necessary documents and information considered in making the Adverse Benefit Determination to the assigned IRO electronically or by telephone or facsimile or any other available expeditious method. The assigned IRO, to the extent the information or documents are available and the IRO considers them appropriate, must consider the information or documents described above under the procedures for Standard External Review. In reaching a decision, the assigned IRO must review the claim de novo and is not bound by any decisions or conclusions reached during the Plan's internal claims and appeals process.

7.2.4 Notice of final External Review decision. The Plan's contract with the assigned IRO must require the IRO to provide notice of the final External Review decision, in accordance with the requirements set forth in paragraph xx above, as expeditiously as the Participant's medical condition or circumstances require, but in no event more than 72 hours after the IRO receives the request for an expedited External Review. If the IRO's notice is not in writing, within 48 hours after the date of providing that notice, the assigned IRO must provide written confirmation of the decision to the Participant and the Plan.

7.2.5 Questions About Your Claims and Appeal Rights: For questions about your rights, these claims procedures, or for assistance, you can contact the Employee Benefits Security Administration at 1.866.444.EBSA (3272).

SEC 8: BENEFITS

A summary of benefits is provided in Appendix A

SEC 9: CONTINUATION OF COVERAGE

9.1 Continuation During Family and Medical Leave Act (FMLA) Leave: The Plan shall at all times comply with FMLA. During any leave taken under FMLA, the Employee will maintain coverage under this Plan on the same conditions as coverage would have been provided if the covered Employee had been continuously employed during the entire leave period.

9.1.1 You may have a right under the FMLA to take unpaid, job-protected FMLA leave in a 12-month period for certain family and medical reasons, including up to 12 weeks of unpaid leave in a 12-month period for the birth of a child or placement of a child for adoption or foster care, for leave related to your own or a family member's serious health condition, or for certain qualifying exigencies related to the deployment of a military member to covered active duty. You may also have a right under the FMLA to take up to 26 weeks of unpaid, job-protected FMLA leave in a single 12-month period to care for a covered servicemember with a serious injury or illness (Military Caregiver Leave).

9.1.2 You may have a right under the FMLA to request that your accrued paid leave be substituted for your FMLA leave. This means that you can request that your accrued paid leave run concurrently with some or all of your unpaid FMLA leave, provided you meet any applicable requirements of our leave policy. Concurrent leave use means the absence will count against both the designated paid leave and unpaid FMLA leave at the same time. If you do not meet the requirements for taking

paid leave, you remain entitled to take available unpaid FMLA leave in the applicable 12-month period. Even if you do not request it, the FMLA allows us to require you to use your available sick, vacation, or other paid leave during your FMLA absence.

9.1.3 During any unpaid portion of FMLA leave, you may be required to continue to make any normal contributions to the cost of the health insurance premiums.

9.1.4 To learn more about your FMLA please contact the Plan Administrator.

9.2 Continuation During USERRA: Participants who are absent from employment because they are in the Uniformed Services may elect to continue their coverage under this Plan for up to 24 months. To continue coverage, Participants must comply with the terms of the Plan, and pay their contributions, if any. In addition, USERRA also requires that, regardless of whether a Participant elected to continue his or her coverage under the Plan, his or her coverage and his or her Dependents' coverage be reinstated immediately upon his or her return to employment, so long as he or she meets certain requirements contained in USERRA. Participants should contact their participating Employer for information concerning their eligibility for USERRA and any requirements of the Plan.

9.3 Continuation During COBRA:

9.3.1 Introduction: The right to this form of continued coverage was created by a Federal law, under the Consolidated Omnibus Budget Reconciliation Act of 1985, as amended ("COBRA"). COBRA Continuation Coverage can become available to Participants when they otherwise would lose their group health coverage. It also can become available to other members of the Participant's family who are covered under the Plan when they otherwise would lose their group health coverage. Under the Plan, Qualified Beneficiaries that elect COBRA Continuation Coverage must pay the entire cost of the coverage, including a reasonable

administration fee. There are several ways coverage will terminate, including the failure of the Participant or their covered Dependents to make timely payment of contributions or premiums. For additional information, Participants should contact the Participating Employer to determine if COBRA applies to him or her and/or his or her covered Dependents. Participants may have other options available when group health coverage is lost. For example, a Participant may be eligible to buy an individual plan through the Health Insurance Marketplace. By enrolling in coverage through the Marketplace, the Participant may qualify for lower costs on his or her monthly premiums and lower out-of-pocket costs. Additionally, the Participant may qualify for a 30-day special enrollment period for another group health plan for which the Participant is eligible (such as a spouse's plan), even if that plan generally doesn't accept late enrollees.

9.3.2 COBRA Continuation Coverage: "COBRA Continuation Coverage" is a continuation of Plan coverage when coverage would otherwise end because of a life event known as a "Qualifying Event." COBRA (and the description of COBRA Continuation Coverage contained in this Plan) does not apply to the following benefits (if available as part of the Employer's plan): life insurance, accidental death and dismemberment benefits and weekly income or long term disability benefits. The aforementioned benefits are not considered for continuation under COBRA. The Plan provides no greater COBRA rights than what COBRA requires – nothing in this Plan is intended to expand the Participant's rights beyond COBRA's requirements.

9.3.3 Qualifying Events: Specific Qualifying Events are listed below. After a Qualifying Event, COBRA Continuation Coverage must be offered to each person who is a "Qualified Beneficiary." A Qualified Beneficiary is someone who is or was covered by the Plan, and has lost or will lose coverage under the Plan due to the occurrence of a Qualifying Event. The Employee and/or

Employee's Dependents could therefore become Qualified Beneficiaries if applicable coverage under the Plan is lost because of the Qualifying Event.

9.3.3 An Employee, who is properly enrolled in this Plan and is a covered Employee, will become a Qualified Beneficiary if he or she loses his or her coverage under the Plan because either one of the following Qualifying Events happens:

9.3.3.1. The hours of employment are reduced.

9.3.3.2. The employment ends for any reason other than gross misconduct.

9.3.4 The spouse of a covered Employee will become a Qualified Beneficiary if he or she loses his or her coverage under the Plan because any of the following Qualifying Events happens:

9.3.4.1 The Employee dies.

9.3.4.2 The Employee's hours of employment are reduced.

9.3.4.3 The Employee's employment ends for any reason other than his or her gross misconduct.

9.3.4.4 The Employee becomes entitled to Medicare benefits (under Part A, Part B, or both).

9.3.4.5 The Employee becomes divorced or legally separated from his or her spouse.

9.3.5 Dependent Children will become Qualified Beneficiaries if they lose coverage under the Plan because any of the following Qualifying Events happens:

9.3.5.1 The parent-covered Employee dies.

9.3.5.2 The parent-covered Employee's hours of employment are reduced.

9.3.5.3 The parent-covered Employee's employment ends for any reason other than his or her gross misconduct.

9.3.5.4 The parent-covered Employee becomes entitled to Medicare benefits (Part A, Part B, or both).

9.3.5.5 The parents become divorced or legally separated.

9.3.5.6 The Child stops being eligible for coverage under the Plan as a Dependent Child.

9.3.6 When the Qualifying Event is the end of employment (for reasons other than gross misconduct), reduction of hours of employment, death of the covered Employee, commencement of a proceeding in bankruptcy with respect to the Employer, or the covered Employee's becoming entitled to Medicare benefits (under Part A, Part B, or both), the Employer must notify the Plan Administrator of the Qualifying Event.

9.3.7 In certain circumstances, the covered Employee or Qualified Beneficiary, in order to protect his or her rights under COBRA, is required to provide notification to the COBRA Administrator in writing, either by U.S. First Class Mail or hand delivery. These circumstances are any of the following:

9.3.7.1 Notice of the occurrence of a Qualifying Event that is a divorce or Legal Separation of a covered Employee (or former Employee) from his or her spouse.

9.3.7.2 Notice of Child's Loss of Dependent Status: Notice of the occurrence of a Qualifying Event that is an individual's ceasing to be eligible as a Dependent Child under the terms of the Plan.

9.3.8 Notice of a Second Qualifying Event: Notice of the occurrence of a second Qualifying Event after a Qualified Beneficiary has become entitled to COBRA Continuation Coverage with a maximum duration of 18 (or 29) months.

9.3.9 Notice Regarding Disability: Notice that a Qualified Beneficiary entitled to receive COBRA Continuation Coverage with a maximum duration of 18 months has been determined by the Social Security Administration ("SSA") to be disabled at any time during the first 60 days of COBRA Continuation Coverage.

9.3.10 Notice Regarding End of Disability: Notice that a Qualified Beneficiary, with respect to whom a notice described above in

has been provided, has subsequently been determined by the SSA to no longer be disabled.

9.3.11 As indicated above, Notification of a Qualifying Event must be made in writing. Notice must be made by submitting the "Notice of Qualifying Event" form and mailing it by U.S. First Class Mail or hand delivery to the COBRA Administrator. This form is available, without charge, from the COBRA Administrator.

9.3.12 Notification must include an adequate description of the Qualifying Event or disability determination. Please see the remainder of this section for additional information.

9.3.13 Notification must be received by the COBRA Administrator at: MBA 7077 E Marilyn Rd Building 1, Scottsdale, AZ 85254

9.3.14 A form of notice is available, free of charge, from the COBRA Administrator and must be used when providing the notice.

9.3.15 Deadline for providing the notice: For Qualifying Events described above, notice must be furnished within 60 days of the latest occurring event set forth below:

9.3.15.1 The date upon which the Qualifying Event occurs.

9.3.15.2 The date upon which the Qualified Beneficiary loses (or would lose) Plan coverage due to a Qualifying Event.

9.3.15.3 The date upon which the Qualified Beneficiary is notified via the Plan's SPD or general notice, and/or becomes aware of their status as a Qualified Beneficiary and/or the occurrence of a Qualifying Event; as well as their subsequent responsibility to comply with the Plan's procedure(s) for providing notice to the COBRA Administrator regarding said status.

9.3.16 As described above, if an Employee or Qualified Beneficiary is determined to be disabled under the Social Security Act, the notice must be delivered no more than 60 days after the latest of:

9.3.16.1 The date of the disability determination by the SSA.

9.3.16.2 The date on which a Qualifying Event occurs.

9.3.16.3 The date on which the Qualified Beneficiary loses (or would lose) coverage under the Plan as a result of the Qualifying Event.

9.3.16.4 The date on which the Qualified Beneficiary is informed, through the furnishing of the Plan's SPD or the general notice, of both the responsibility to provide the notice and the Plan's procedures for providing such notice to the COBRA Administrator.

9.3.16.5 In any event, this notice must be provided within the first 18 months of COBRA Continuation Coverage.

9.3.17 For a change in disability status described above, the notice must be furnished by the date that is 30 days after the later of:

9.3.17.1 The date of the final determination by the SSA that the Qualified Beneficiary is no longer disabled.

9.3.17.2 The date on which the Qualified Beneficiary is informed, through the furnishing of the Plan's SPD or the general notice, of both the responsibility to provide the notice and the Plan's procedures for providing such notice to the COBRA Administrator.

9.3.17.3 The notice must be postmarked (if mailed), or received by the COBRA Administrator (if hand delivered), by the deadline set forth above. If the notice is late, the opportunity to elect or extend COBRA Continuation Coverage is lost, and if the person is electing COBRA Continuation Coverage, his or her coverage under the Plan will terminate on the last date for which he or she is eligible under the terms of the Plan, or if the person is extending COBRA Continuation Coverage, such Coverage will end on the last day of the initial 18-month COBRA coverage period.

9.3.18 Any individual who is the covered Employee (or former Employee) with respect to a Qualifying Event, or any representative acting on behalf of the covered Employee (or

former Employee) or Qualified Beneficiary, may provide the notice. Notice by one individual shall satisfy any responsibility to provide notice on behalf of all related Qualified Beneficiaries with respect to the Qualifying Event.

9.3.19 Required Contents of the Notice: After receiving a notice of a Qualifying Event, the Plan must provide the Qualified Beneficiary with an election notice, which describes their rights to COBRA Continuation Coverage and how to make such an election. The notice must contain the following information:

9.3.19.1 Name and address of the covered Employee or former Employee.

9.3.19.2 Name of the Plan and the name, address, and telephone number of the Plan's COBRA administrator.

9.3.19.3 Identification of the Qualifying Event and its date (the initial Qualifying Event and its date if the Qualifying Participant is already receiving COBRA Continuation Coverage and wishes to extend the maximum coverage period).

9.3.19.4 A description of the Qualifying Event (for example, divorce, Legal Separation, cessation of Dependent status, entitlement to Medicare by the covered Employee or former Employee, death of the covered Employee or former Employee, disability of a Qualified Beneficiary or loss of disability status):

9.3.19.5 In the case of a Qualifying Event that is divorce or Legal Separation, name(s) and address(es) of spouse and Dependent Child or Children covered under the Plan, date of divorce or Legal Separation, and a copy of the decree of divorce or Legal Separation.

9.3.19.6 In the case of a Qualifying Event that is Medicare entitlement of the covered Employee or former Employee, date of entitlement, and name(s) and

address(es) of spouse and Dependent Child or Children covered under the Plan.

9.3.19.7 In the case of a Qualifying Event that is a Dependent Child's cessation of Dependent status under the Plan, name and address of the Child, reason the Child ceased to be an eligible Dependent (for example, attained limiting age).

9.3.19.8 In the case of a Qualifying Event that is the death of the covered Employee or former Employee, the date of death, and name(s) and address(es) of spouse and Dependent Child or Children covered under the Plan.

9.3.19.9 In the case of a Qualifying Event that is disability of a Qualified Beneficiary, name and address of the disabled Qualified Beneficiary, name(s) and address(es) of other family members covered under the Plan, the date the disability began, the date of the SSA's determination, and a copy of the SSA's determination.

9.3.19.10 In the case of a Qualifying Event that is loss of disability status, name and address of the Qualified Beneficiary who is no longer disabled, name(s) and address(es) of other family members covered under the Plan, the date the disability ended and the date of the SSA's determination.

9.3.19.11 Identification of the Qualified Beneficiaries (by name or by status).

9.3.19.12 An explanation of the Qualified Beneficiaries' right to elect continuation coverage.

9.3.19.13 The date coverage will terminate (or has terminated) if continuation coverage is not elected.

9.3.19.14 How to elect continuation coverage.

9.3.19.15 What will happen if continuation coverage isn't elected or is waived.

9.3.19.16 What continuation coverage is available, for how long, and (if it is for less than 36 months), how it can be extended for disability or second qualifying events.

9.3.19.17 How continuation coverage might terminate early.

9.3.19.18 Premium payment requirements, including due dates and grace periods.

9.3.19.19 A statement of the importance of keeping the Plan Administrator informed of the addresses of Qualified Beneficiaries.

9.3.19.20 A statement that the election notice does not fully describe COBRA or the plan and that more information is available from the Plan Administrator and in the SPD.

9.3.19.21 A certification that the information is true and correct, a signature and date.

9.3.19.22 If a copy of the decree of divorce or Legal Separation or the SSA's determination cannot be provided by the deadline for providing the notice, complete and provide the notice, as instructed, by the deadline and submit the copy of the decree of divorce or Legal Separation or the SSA's determination within 30 days after the deadline. The notice will be timely if done so. However, no COBRA Continuation Coverage, or extension of such Coverage, will be available until the copy of the decree of divorce or Legal Separation or the SSA's determination is provided.

9.3.19.23 If the notice does not contain all of the required information, the COBRA Administrator may request additional information. If the individual fails to provide such information within the time period specified by the COBRA Administrator in the request, the COBRA Administrator may reject the notice if it does not contain enough information for the COBRA Administrator to identify the plan, the covered Employee (or former Employee), the Qualified Beneficiaries, the Qualifying

Event or disability, and the date on which the Qualifying Event, if any, occurred.

9.3.20 Electing COBRA Continuation Coverage: Complete instructions on how to elect COBRA Continuation Coverage will be provided by the COBRA Administrator within 14 days of receiving the notice of the Qualifying Event. The individual then has 60 days in which to elect COBRA Continuation Coverage. The 60-day period is measured from the later of the date coverage terminates or the date of the notice containing the instructions. If COBRA Continuation Coverage is not elected in that 60-day period, then the right to elect it ceases.

9.3.21 Each Qualified Beneficiary will have an independent right to elect COBRA Continuation Coverage. Covered Employees may elect COBRA Continuation Coverage on behalf of all other Qualified Beneficiaries, including their spouses, and parents or a legal guardian may elect COBRA Continuation Coverage on behalf of their Children.

9.3.22 In the event that the COBRA Administrator determines that the individual is not entitled to COBRA Continuation Coverage, the COBRA Administrator will provide to the individual an explanation as to why he or she is not entitled to COBRA Continuation Coverage.

9.3.23 The maximum time period shown below shall dictate for how long COBRA Continuation Coverage will be available. The maximum time period for coverage is based on the type of the Qualifying Event and the status of the Qualified Beneficiary. Multiple Qualifying Events that may be combined under COBRA will not ordinarily continue coverage for more than 36 months beyond the date of the original Qualifying Event. When the Qualifying Event is “entitlement to Medicare,” the 36-month continuation period is measured from the date of the original Qualifying Event. For all other Qualifying Events, the continuation

period is measured from the date of the Qualifying Event, not the date of loss of coverage.

9.3.24 When the Qualifying Event is the death of the covered Employee (or former Employee), the covered Employee's (or former Employee's) becoming entitled to Medicare benefits (under Part A, Part B, or both), a divorce or Legal Separation, or a Dependent Child's losing eligibility as a Dependent Child, COBRA Continuation Coverage lasts for up to a total of 36 months.

9.3.25 When the Qualifying Event is the end of employment or reduction of the covered Employee's hours of employment, and the covered Employee became entitled to Medicare benefits less than 18 months before the Qualifying Event, COBRA Continuation Coverage for Qualified Beneficiaries other than the covered Employee lasts until 36 months after the date of Medicare entitlement. For example, if a covered Employee becomes entitled to Medicare eight months before the date on which his or her employment terminates, COBRA Continuation Coverage for his or her spouse and Children can last up to 36 months after the date of Medicare entitlement, which is equal to 28 months after the date of the Qualifying Event (thirty-six months minus eight months).

9.3.26 Otherwise, when the Qualifying Event is the end of employment (for reasons other than gross misconduct) or reduction of the covered Employee's hours of employment, COBRA Continuation Coverage generally lasts for only up to a total of 18 months. There are two ways in which this eighteen-month period of COBRA Continuation Coverage can be extended.

9.4 Disability can extend the 18-month period of continuation coverage for a Qualifying Event that is a termination of employment or reduction of hours, if an Employee or anyone in an Employee's family covered under the Plan is determined by the Social Security Administration ("SSA") to be disabled, and the Employee notifies the COBRA Administrator. The Employee and his or her Dependents may thereby be entitled to an

additional 11 months of COBRA Continuation Coverage, for a total of 29 months, if the disability started at some time before the 60th day of COBRA Continuation Coverage and lasts at least until the end of the 18-month period of COBRA Continuation Coverage. The Plan can charge 150% of the premium cost for the extended period of coverage.

9.5 If an Employee's family experiences another Qualifying Event while receiving 18 months of COBRA Continuation Coverage, Dependents may receive up to 18 additional months of COBRA Continuation Coverage, for a maximum of 36 months, if notice of the second Qualifying Event is provided to the Plan Administrator or COBRA Administrator in accordance with the procedures set forth herein. This extension may be applicable to the Employee's death, Medicare Parts A and/or B eligibility, divorce or Legal Separation, or a loss of Dependent status under the terms of the Plan if the event would have also caused the spouse or Dependent Child to lose coverage under the Plan regardless of whether the first Qualifying Event had occurred.

9.6 COBRA establishes required periods of coverage for continuation health benefits. A plan, however, may provide longer periods of coverage beyond those required by COBRA. COBRA Qualified Beneficiaries generally are eligible for group coverage during a maximum of 18 months after Qualifying Events arising due to employment termination or reduction of hours of work. Certain Qualifying Events, or a second Qualifying Events during the initial period of coverage, may permit a Qualified Beneficiary to receive a maximum of 36 months of coverage.

9.7 It is not necessary that COBRA Continuation Coverage be in effect for the maximum period of time, as set forth herein. COBRA Continuation Coverage may conclude prior to the latest possible date if the Employer ceases to provide a group health plan to any Employee; the Qualified Beneficiary fails to make timely payment of any required contributions or premium; the Qualified Beneficiary gains coverage under another group health plan (as an Employee or otherwise) or becomes entitled to either Medicare Part A or Part B (whichever comes first); and/or any other event occurs which enables the Plan Administrator to terminate

coverage without offering COBRA Continuation Coverage (such as the commission of fraud by the Qualified Beneficiary and/or their Dependent). COBRA Continuation Coverage shall be extended to the first day of the month, 30 days (or more) subsequent to the date upon which the SSA determined that the Qualified Beneficiary is no longer disabled.

9.8 The cost of the elected COBRA Continuation Coverage must be paid within 45 days of its election. Payments will then be subsequently due on the first day of each month. COBRA Continuation Coverage will be canceled and will not be reinstated if any payment is made late; however, the Plan Administrator must allow for a 30-day grace period during which a late payment may still be made without the loss of COBRA Continuation Coverage.

9.9 The Trade Preferences Extension Act of 2015 has extended certain provisions of the Trade Reform Act of 2002, which created a special COBRA right applicable to certain employees who have been terminated or experienced a reduction of hours and who qualify for a "trade readjustment allowance" or "alternative trade adjustment assistance." These individuals can either take a Health Coverage Tax Credit (HCTC) or get advance payment of the applicable percentage of premiums paid for qualified health insurance coverage, including COBRA continuation coverage. These individuals are also entitled to a second opportunity to elect COBRA coverage for themselves and certain family members (if they did not already elect COBRA coverage). This election must be made within the 60-day period that begins on the first day of the month in which the individual becomes eligible for assistance under the Trade Reform Act of 2002. However, this election may not be made more than six months after the date the individual's group health plan coverage ends. A Participant's eligibility for subsidies under the Trade Preferences Extension Act of 2015 affects his or her eligibility for subsidies that provide premium assistance for coverage purchased through the Health Insurance Marketplace. For each coverage month, a Participant must choose one or the other, and if he

or she receives both during a tax year, the IRS will reconcile his or her eligibility for each subsidy through his or her individual tax return. Participants may wish to consult their individual tax advisors concerning the benefits of using one subsidy or the other.

Participants may contact the Plan Administrator for additional information or if they have any questions, they may call the Health Coverage Tax Credit Customer Contact Center toll-free at 1-866-628-4282. TTD/TTY callers may call toll-free at 1-866-626-4282. More information about the Trade Reform Act is available at www.doleta.gov/tradeact; for information about the Health Coverage Tax Credit (HCTC), please see: https://www.irs.gov/Credits-&-Deductions/Individuals/HCTC. Please contact the COBRA Administrator, with any questions about the Plan and COBRA Continuation Coverage at the following: MBA 7077 E Marilyn Rd Building 1, Scottsdale, AZ 85254

9.10 Questions concerning the Plan or COBRA continuation coverage rights should be addressed to the contact or contacts identified below. For more information about a Participant's rights under the Employee Retirement Income Security Act (ERISA), including COBRA, the Affordable Care Act, and other laws affecting group health plans, contact the nearest Regional or District Office of the U.S. Department of Labor's Employee Benefits Security Administration (EBSA) or visit https://www.dol.gov/agencies/ebsa. (Addresses and phone numbers of Regional and District EBSA Offices are available through EBSA's website.) For more information about the Marketplace, visit www.HealthCare.gov.

9.11 Important information may be distributed by mail. In order to protect the rights of the Employee's family, the Employee should keep the COBRA Administrator (who has been previously identified in this section) informed of any changes in the addresses of family members.

SEC 10: ADDITIONAL BENEFITS

10.1 MENTAL HEALTH AND SUBSTANCE USE DISORDER PARITY (If the Plan is not subject to an Increased Cost Exemption): The Plan may not apply more restrictive financial requirements or treatment limitations to mental health or substance use disorder benefits in any classification than the predominant limitations applied to substantially all of the medical and surgical benefits in any classification.

10.1.1 The criteria for medical necessity determinations made under any Plan with respect to mental health or substance use disorder benefits shall be made available by the Plan Administrator (in accordance with the Wellstone Act) to any current or potential Participant upon request.

10.1.2 The reason for any denial under the Plan of reimbursement or payment for services with respect to mental health or substance use disorder benefits in the case of any Participant shall, on request or as otherwise required under the Wellstone Act, be made available by the Plan Administrator to the Participant in accordance with the claims procedures applicable to the Plan.

10.1.3 The Plan shall be operated and construed in all respects in compliance with the MHPA and the Wellstone Act.

10.1.4 "Mental health benefits" and "substance use disorder benefits" mean benefits with respect to items or services for mental health conditions and substance use disorders, respectively, and shall be as defined pursuant to applicable state and federal law and consistent with generally recognized independent standards of current medical practice.

10.2 MICHELLE'S LAW

10.2.1 If an Eligible Dependent is a dependent child on a medically necessary leave of absence, the coverage under the Plan shall not end until the earlier of (1) the date that is one year following the first day of the medically necessary leave of absence or (2) the date on which

coverage under the Plan would otherwise terminate under the terms of the Plan. For purposes of Michelle's Law:

10.2.2 "dependent child" means the child of a Participant who is an Eligible Dependent and was enrolled in Plan, on the basis of being a student at a postsecondary educational institution (including an institution of higher education as described in Section 102 of the Higher Education Act of 1965), immediately before the first day of a medically necessary leave of absence.

10.2.3 A "medically necessary leave of absence" means a leave of absence of a dependent child from a postsecondary educational institution (including an institution of higher education as described in Section 102 of the Higher Education Act of 1965), or any other change in enrollment of such child at such an institution, that:

10.2.3.1 Commences while such dependent child is suffering from a serious illness or injury;

10.2.3.2 Is medically necessary; and

10.2.3.3 Causes such dependent child to lose student status for purposes of coverage under the Plan.

10.2.3.4 No leave of absence (or other change of enrollment) shall be considered a medically necessary leave of absence unless the Plan receives written certification by a treating physician of the dependent child that the dependent child is suffering from a serious illness or injury and that the leave of absence (or other change of enrollment) is medically necessary.

<u>10.3 WOMEN'S HEALTH AND CANCER RIGHTS ACT (MASTECTOMIES)</u>: If you have had or are going to have a mastectomy, you may be entitled to certain benefits under the Women's Health and Cancer Rights Act of 1998 (WHCRA). For individuals receiving mastectomy-related benefits, coverage will be provided in a manner determined in consultation with the attending physician and the patient, for:

10.3.1 All stages of reconstruction of the breast on which the mastectomy was performed;

10.3.2 Surgery and reconstruction of the other breast to produce a

symmetrical appearance;

10.3.3 Prostheses; and Treatment of physical complications of the mastectomy, including lymphedema.

10.3.4 These benefits will be provided subject to the same deductibles and coinsurance applicable to other medical and surgical benefits provided under this plan. If you would like more information on WHCRA benefits, call your plan administrator at 480-576-7118.

SEC 11: HEALTH INFORMATION PRIVACY (HIPAA) RIGHTS

11.1 HIPAA requires group health plans to protect the confidentiality of your private health information. The Plan and the Company will not use or further disclose information that is protected by HIPAA ("Protected Health Information") except as necessary for treatment, payment, health plan operations, and Plan administration, or as otherwise permitted or required by applicable law. In particular, the Plan will not, without written authorization from you, use or disclose your Protected Health Information for employment-related actions and decisions or in connection with any other benefit or employee benefit plan of the Company. In addition, the Plan requires all of its business associates (that is, service providers who help us administer the Plan) to also observe HIPAA's privacy rules.

11.2 Under HIPAA, you have certain rights with respect to your Protected Health Information, including certain rights to see and copy the information, receive an accounting of certain disclosures of the information and, under certain circumstances, amend the information. You also have the right to file a complaint with the Plan or with the Secretary of the US Department of Health and Human Services if you believe your rights under HIPAA have been violated.

11.3 To the extent required by applicable law, the Plan will maintain a privacy notice, which provides a complete description of your rights under HIPAA's privacy rules.

11.4 For a copy of the notice, please contact the Plan Administrator. If you have questions about the privacy of your health information, or if you wish to file a complaint under HIPAA, you should contact the Company's privacy officer.

SEC 12: RECOVERY/SUBROGATION/REIMBURSEMENT

12.1 Refund of overpayments

12.1.1 Whenever a payment has been made under the Plan in a total amount, at any time, in excess of the maximum amount payable under the Plan's provision ("Overpayment"), you or any other Covered Person must refund to the Plan the applicable Overpayment and help the Plan obtain the refund of the Overpayment from another person or organization. This includes any Overpayments resulting from retroactive awards received from any source, fraud, or any error made in processing your claim.

12.1.2 In case of a recovery from a source other than the Plan, Overpayment recovery will not be more than the amount of the payment. An Overpayment also occurs when payment is made from the Plan that should have been made under another group plan. In that case, the Plan may recover the payment from one or more of the following: any other insurance company, any other organization, or any person to or for whom payment was made.

12.1.3 The Plan may, at its option, recover the Overpayment by reducing or offsetting against any future benefits payable to the Covered Person or his/her survivors; stopping future benefit payments that would otherwise be due under the Plan (payments may continue when the Overpayment has been recovered); or demanding an immediate refund of the Overpayment from the Covered Person.

12.2 Reimbursement: This section applies when you or any other Covered Person recovers damages — by settlement, verdict, or otherwise — for an injury, sickness, or other condition.

12.2.1 If the Covered Person has made — or in the future may make — such a recovery, including a recovery from an insurance carrier, the Plan will not cover either the reasonable value of the services to treat such an injury or illness or the treatment of such an injury or illness.

12.2.2 However, if the Plan does pay for or provide benefits for such an injury, sickness, or other condition, the Covered Person or the legal representatives, estate, or heirs of the Covered Person will promptly reimburse the Plan for all recovery amounts (whether or not characterized as related to medical expenses) from any settlement, verdict, or insurance proceeds received by the Covered Person (or by the legal representatives, estate, or heirs of the Covered Person) to the extent that medical benefits have been paid for or provided by the Plan to the Covered Person.

12.2.3 If the Covered Person receives payment from a third party or his or her insurance company as a result of an injury or harm due to the conduct of another party and the Covered Person has received benefits from the Plan, the Plan must be reimbursed first. In other words, the Covered Person's recovery from a third party may not compensate the Covered Person fully for all the financial expenses incurred because acceptance of benefits from the Plan constitutes an agreement to reimburse the Plan for any benefits the Covered Person receives.

12.2.4 The Covered Person also must take any reasonably necessary action to protect the Plan's subrogation and reimbursement rights. That means that by accepting benefits from the Plan, the Covered Person agrees to notify the Plan Administrator if and when the Covered Person institutes a lawsuit or other action or enters into settlement negotiations with another party (including his or her

insurance company) in connection with or related to the conduct of another party.

12.2.5 The Covered Person also must cooperate with the Plan Administrator's reasonable requests concerning the Plan's subrogation and reimbursement rights and must keep the Plan Administrator informed of any important developments in his or her action. The Covered Person also agrees that the Plan Administrator may withhold any future benefits paid by this Plan or any other disability or health plan maintained by the Company to the extent necessary to reimburse this Plan under the Plan's subrogation or reimbursement rights.

12.2.6 To secure the rights of the Plan under this section, each Covered Person hereby:

12.2.6.1. Agrees that, with respect to any amounts paid by the Plan for the Covered Person's injury, sickness, or other condition, the Plan will be reimbursed in full from any policy proceeds, judgment, or settlement before any amounts from such policy proceeds, judgment, or settlement (including attorney's fees incurred by the Covered Person) are paid to any other person, regardless of the manner in which the recovery is structured or worded;

12.2.6.2. Grants to the Plan a first-priority lien against the proceeds of any such settlement, verdict, or other amounts received by the Covered Person to the extent of all benefits provided in an effort to make the Plan whole;

12.2.6.3. Assigns to the Plan any benefits the Covered Person may have under any automobile policy or other coverage; the Covered Person shall sign and deliver, at the request of the Plan or its agents, any documents needed to protect such lien or to effect such assignment of benefits;

12.2.6.4. Agrees that the costs of legal representation retained by the Covered Person shall be borne solely by the Covered Person; and

12.2.6.5. Will cooperate with the Plan and its agents and will:

12.2.6.6. Sign and deliver such documents as the Plan or its agents reasonably request to protect the Plan's right of reimbursement;

12.2.6.7. Provide any relevant information; and

12.2.6.8. Take such actions as the Plan or its agents reasonably request to assist the Plan in making a full recovery of the value of the benefits provided.

12.2.6.9. If the Covered Person does not sign and deliver any such documents for any reason (including, but not limited to, the fact that the Covered Person was not given an agreement to sign or is unable or refuses to sign), the Plan Administrator, in its sole discretion, may or may not advance benefits to the Covered Person under the Plan.

12.2.6.10. If the Plan Administrator has advanced benefits, it has the right to subrogation and reimbursement whether or not the Covered Person has signed the agreement. The Covered Person shall not take any action that prejudices the Plan's right of reimbursement.

12.3 Subrogation: This section applies when another party is, or may be considered, liable for:

12.3.1 a Covered Person's injury, sickness, or other condition (including insurance carriers that are so liable) and

12.3.2 the Plan has provided or paid for benefits.

12.3.3 The Plan is subrogated to all the rights of the Covered Person against any party, including any insurance carrier, liable for the Covered Person's injury or illness or for the payment for the medical treatment of such injury or occupational illness to the extent of the value of the medical benefits provided to the Covered Person under the Plan.

12.3.4 The Plan may assert this right independently of the Covered Person.

12.3.5 The Covered Person is obligated to cooperate with the Plan and its agents to protect the Plan's subrogation rights. Cooperation means providing the Plan or its agents with relevant information requested by them; signing and delivering such documents as the Plan or its

agents reasonably request to secure the Plan's subrogation claim; and obtaining the consent of the Plan or its agents before releasing any party from liability for payment.

12.3.6 If the Covered Person enters into litigation or settlement negotiations regarding the obligations of other parties, the Covered Person must not prejudice, in any way, the subrogation rights of the Plan under this section. Further, the Covered Person agrees to notify the Plan Administrator if and when the Covered Person institutes la lawsuit or other action or enters into settlement negotiations with another party (including his or her insurance company) in connection with or related to the conduct of another party.

12.3.7 The Covered Person agrees that, with respect to any amounts paid by the Plan for a Covered Person's injury, sickness, or other condition, the Plan will be reimbursed in full from any policy proceeds, judgment, or settlement before any amounts from such policy proceeds, judgment, or settlement (including attorney's fees incurred by the Covered Person) are paid to any other person, regardless of the manner in which the recovery is structured or worded.

12.3.8 The costs of legal representation retained by the Plan in matters related to subrogation shall be borne solely by the Plan. The costs of legal representation retained by the Covered Person shall be borne solely by the Covered Person.

12.4 Misrepresentation:

12.4.1 If a Participant or any other person makes any intentional misrepresentation or uses fraudulent means in applying for coverage under the Plan, making a change in his or her existing coverage election under the Plan, or filing a claim for benefits under the Plan, his or her coverage under the Plan will be subject to immediate termination, recoupment by the Plan of erroneously paid expenses based on the misrepresentation or fraud, and other

remedies available to the Plan Administrator at law and in equity subject to and in accordance with applicable law.

12.4.2 The Plan Administrator has the sole and absolute discretion and authority to verify the initial and continuing eligibility for participation and benefits under the Plan of any person, including any child, spouse, domestic partner, or dependent of an employee, by requesting proof of such eligibility including, as applicable and without limitation, tax returns, phone records, email records marriage certificates, birth certificates, proof of residence, proof of domestic partnership, or other documentation deemed appropriate by the Plan Administrator. The Plan Administrator has the sole and absolute discretion to refuse enrollment or continuing participation in the Plan to any individual who refuses or otherwise fails to provide such proof of eligibility.

SEC 13: MISCELLANEOUS

13.1 No Contract: Nothing contained in the Plan nor any action taken hereunder shall be construed as a contract of employment or as giving any Eligible Employee any right to be retained in the employ of the Company.

13.2 Pronouns: A pronoun or adjective in the masculine gender includes the feminine gender, and the singular includes the plural, unless the context clearly indicates otherwise.

13.3 Headings and numbers: in this Plan are included for convenience of reference only, and if there shall be any conflict between any of the numbers and headings and the text of the Plan, the text shall control.

13.4 Information requested of Participants: Participants shall provide the Plan Administrator with such information and evidence, and shall sign such documents, as may reasonably be requested from time-to-time for the purpose of administration of the Plan.

13.5 Effect of Amendment: Neither the establishment of the Plan nor any amendment thereof will be construed as giving to any Participant or other person any legal or equitable right against the Plan Administrator or the Company, and in no event will the terms of employment or service of any Eligible Employee be modified or in any way affected hereby.

13.6 Participant Benefits: The Plan is maintained for the exclusive benefit of the Participants.

13.7 No Vesting of Rights: No employee of the Company, whether or not a Participant in, or eligible to participate in, the Plan, nor any Eligible Dependent, shall at any time have any vested rights to benefits provided under the Plan.

13.8 Benefits Source: The benefits provided hereunder will be paid solely from the general assets of the Company. Nothing herein will be construed to require the Company or the Plan Administrator to maintain any fund or segregate any amount for the benefit of any Participant in this Plan, and no Participant or other person shall have any claim against, right to, or security or other interest in, any fund, account, or asset of the Company from which any self-insured benefit payment under the Plan may be made.

13.9 Controlling State Law: To the extent not preempted by ERISA or any other federal statutes or regulations, this Plan shall be governed by, and construed in accordance with, the laws of Texas.

13.10 Vendor Services: The Company does not guarantee and is not responsible for the nature or quality of the goods or services provided through any health care provider or program.

13.11 Verification of Network Coverage: To verify whether a particular service is covered under the Plan, please contact the applicable provider or administrator set forth in Appendix A: Summary of Benefits and seek written verification of the coverage determination.

13.12 MEC: This Plan provides "minimum essential coverage" within the meaning of Section 5000A(f) of the Code.

13.13 Clerical Error/Delay

13.13.1 Any clerical error by the Plan Administrator or an agent of the Plan Administrator in keeping pertinent records or a delay in making any changes to such records will not invalidate coverage otherwise validly in force or continue coverage validly terminated.

13.13.2 Contributions made in error by Participants due to such clerical error will be returned to the Participant; coverage will not be inappropriately extended.

13.13.3 Contributions that were due but not made, in error and due to such clerical error will be owed immediately upon identification of said clerical error.

13.13.4 Failure to so remedy amounts owed may result in termination of coverage.

13.13.5 Effective Dates, waiting periods, deadlines, rules, and other matters will be established based upon the terms of the Plan, as if no clerical error had occurred.

13.13.6 An equitable adjustment of contributions will be made when the error or delay is discovered.

13.13.7 If, an overpayment occurs in a Plan reimbursement amount, the Plan retains a contractual right to the overpayment. The person or institution receiving the overpayment will be required to return the incorrect amount of money. In the case of a Plan Participant, the amount of overpayment may be deducted from future benefits payable.

13.14 Conformity With Applicable Laws

13.14.1 Any provision of this Plan that is contrary to any applicable law, equitable principle, regulation or court order (if such a court is of competent jurisdiction) will be interpreted to comply with said law, or, if it cannot be so interpreted, shall be automatically amended to satisfy the law's minimum requirement, including, but

not limited to, stated maximums, Exclusions, or statutes of limitations.

13.14.2 It is intended that the Plan will conform to the requirements of ERISA, as it applies to Employee welfare plans, as well as any other applicable law.

13.15 Fraud

13.15.1 Under this Plan, coverage may be retroactively canceled or terminated (rescinded) if a Participant acts fraudulently or intentionally makes material misrepresentations of fact.

13.15.2 It is a Participant's responsibility to provide accurate information and to make accurate and truthful statements, including information and statements regarding family status, age, relationships, etc.

13.15.3 It is also a Participant's responsibility to update previously provided information and statements. Failure to do so may result in coverage of Participants being canceled, and such cancellation may be retroactive.

13.15.4 If a Participant, or any other entity, submits or attempts to submit a claim for or on behalf of a person who is not a Participant of the Plan; submits a claim for services or supplies not rendered; provides false or misleading information in connection with enrollment in the Plan; or provides any false or misleading information to the Plan as it relates to any element of its administration; that shall be deemed to be fraud. If a Participant is aware of any instance of fraud, and fails to bring that fraud to the Plan Administrator's attention, that shall also be deemed to be fraud.

13.15.5 Fraud will result in immediate termination of all coverage under this Plan for the Participant and their entire Family Unit of which the Participant is a member.

13.15.6 A determination by the Plan that a rescission is warranted will be considered an Adverse Benefit Determination for purposes of review and appeal.

13.15.7 A Participant whose coverage is being rescinded will be provided a 30-day notice period as described under the Affordable Care Act (ACA) and regulatory guidance.

13.15.8 Claims Incurred after the retroactive date of termination shall not be further processed and/or paid under the Plan.

13.15.9 Claims Incurred after the retroactive date of termination that were paid under the Plan will be treated as erroneously paid claims under this Plan.

13.16 Headings: The headings used in this Plan Document are used for convenience of reference only. Participants are advised not to rely on any provision because of the heading.

13.17 Word Usage: Wherever any words are used herein in the singular or plural, they shall be construed as though they were in the plural or singular, as the case may be, in all cases where they would so apply.

13.18 No Waiver or Estoppel: All parts, portions, provisions, conditions, and/or other items addressed by this Plan shall be deemed to be in full force and effect, and not waived, absent an explicit written instrument expressing otherwise; executed by the Plan Administrator. Absent such explicit waiver, there shall be no estoppel against the enforcement of any provision of this Plan. Failure by any applicable entity to enforce any part of the Plan shall not constitute a waiver, either as it specifically applies to a particular circumstance, or as it applies to the Plan's general administration. If an explicit written waiver is executed, that waiver shall only apply to the matter addressed therein, and shall be interpreted in the most narrow fashion possible.

13.19 Plan Contributions:

13.19.1 The benefits provided hereunder are paid pursuant to a contract between the Plan Sponsor and the Plan Administrator which designates specific dollar amount the Plan Sponsor is required to contribute in full.

13.19.2 The Plan Administrator shall, from time to time, evaluate the funding method of the Plan and determine the amount to be contributed by the Plan Sponsor and the amount to be contributed (if any) by each Participant.

13.19.3 The Plan Sponsor shall fund the Plan in a manner consistent with the provisions of the Internal Revenue Code, ERISA, and such other laws and regulations as shall be applicable to the end that the Plan shall be funded on a lawful and sound basis. The manner and means by which the Plan is funded shall be solely determined by the Plan Sponsor, to the extent allowed by applicable law.

13.19.4 Notwithstanding any other provision of the Plan, the Plan Sponsor's obligation to pay claims otherwise allowable under the terms of the Plan shall be limited to its obligation to make contributions to the Plan as set forth in the preceding paragraphs. Payment of said claims in accordance with these procedures shall discharge completely the Plan Sponsor's obligation with respect to such payments.

13.19.4. Nothing herein will be construed to require the Company, Sponsor or Plan Administrator to maintain any fund or segregate any amount for the benefit of any Participant in this Plan, and no Participant or other person shall have any claim against, right to, or security or other interest in, any fund, account or asset of the company from which any self-funded benefit payment under the Plan may be made.

13.19.5. In the event that the Company terminates the Plan, then as of the effective date of termination, the Employer and eligible Employees shall have no further obligation to make additional contributions to the Plan and the Plan shall have no obligation to pay claims Incurred after the termination date of the Plan.

13.9 Right to Receive and Release Information: The Plan Administrator may, without notice to or consent of any person, release to or obtain any information from any insurance company or other organization or person any information regarding coverage, expenses, and benefits which the

Plan Administrator, at its sole discretion, considers necessary to determine and apply the provisions and benefits of this Plan. In so acting, the Plan Administrator shall be free from any liability that may arise with regard to such action. Any Participant claiming benefits under this Plan shall furnish to the Plan Administrator such information as requested and as may be necessary to implement this provision.

13.10 Written Notice: Any written notice required under this Plan which, as of the Effective Date, is in conflict with the law of any governmental body or agency which has jurisdiction over this Plan shall be interpreted to conform to the minimum requirements of such law.

13.11 Right of Recovery: In accordance with the Recovery of Payments provision, whenever payments have been made by this Plan in a total amount, at any time, in excess of the maximum amount of benefits payable under this Plan, the Plan shall have the right to recover such payments, to the extent of such excess, from any one or more of the following as this Plan shall determine: any person to or with respect to whom such payments were made, or such person's legal representative, any insurance companies, or any other individuals or organizations which the Plan determines are responsible for payment of such amount, and any future benefits payable to the Participant or his or her Dependents. See the Recovery of Payments provision for full details.

13.12 Statements:

13.12.1 All statements made by the Company or by a Participant will, in the absence of fraud, be considered representations and not warranties, and no statements made for the purpose of obtaining benefits under this document will be used in any contest to avoid or reduce the benefits provided by the document unless contained in a written application for benefits and a copy of the instrument containing such representation is or has been furnished to the Participant.

13.12.2 Any Participant who knowingly and with intent to defraud the Plan, files a statement of claim containing any materially false

information, or conceals for the purpose of misleading, information concerning any material fact, commits a fraudulent act. The Participant may be subject to prosecution by the United States Department of Labor. Fraudulently claiming benefits may be punishable by a substantial fine, imprisonment, or both.

13.13 Protection Against Creditors: To the extent this provision does not conflict with any applicable law, no benefit payment under this Plan shall be subject in any way to alienation, sale, transfer, pledge, attachment, garnishment, execution or encumbrance of any kind, and any attempt to accomplish the same shall be void. If the Plan Administrator shall find that such an attempt has been made with respect to any payment due or to become due to any Participant, the Plan Administrator in its sole discretion may terminate the interest of such Participant or former Participant in such payment. And in such case the Plan Administrator shall apply the amount of such payment to or for the benefit of such Participant or former Participant, his or her spouse, parent, adult Child, guardian of a minor Child, brother or sister, or other relative of a Dependent of such Participant or former Participant, as the Plan Administrator may determine, and any such application shall be a complete discharge of all liability with respect to such benefit payment. However, at the discretion of the Plan Administrator, benefit payments may be assigned to health care Providers.

APPENDIX A

GoodHealth Distribution Partners

GHDP Summary

Features/Benefits	GHW	GH1	GH2	GH3	GH4	GH5
Telemedicine	✓	✓	✓	✓	✓	✓
BestChoiceRX Group Discount Program	✓	✓				
BestChoiceRX (Accute & Wellness $0 Co-pay Maintenance Generic $5 Co-Pay)			✓			
BestChoiceRX Plus - Includes non-preferred Generic & Brand with a co-pay up to a maximum of $150/mo				✓	✓	✓
Preventative Care[2]	✓	✓	✓	✓	✓	✓
Annual Wellness Exam	✓				✓	✓
Specialty Drugs - Prescription Assistance Program	✓	✓	✓	✓	✓	✓
Healthcare Ninja		✓	✓	✓	✓	✓
Primary Care Physician Visits[1,2]		3	4	4	4	5
Specialist / UC Visit[1,2]		1	2	4	4	5
Hospital Indemnity Benefit[3]		✓	✓	✓	✓	✓
Emergency Room Services[3]					✓	✓
Ambulance Services[3]					✓	✓
In-Out patient surgery[3]					✓	✓

Dental	1500 Plan	3000 Plan	5000 Plan
Individual & Family Dental Plans	$1,500 annual max per person	$3,000 annual max per person	$5,000 annual max per person
Dental/Vision Bundle	**Self-funded Dental & employees can choose from 99,000+ vision providers at private practices & national, regional, and local retail chains.**		

(1) All sickness benefits are subject to a 30-day waiting period before benefits are payable under the plan.
(2) Outpatient physician services and wellness benefits are subject to in-network providers only. Inpatient Hospital indemnity benefits are not.
(3) Hospitalization benefits are not payable for a Pre-Existing Condition as defined in Section 2.56 Definitions for the first Twelve [12] Months of coverage

This Plan does not cover services unless listed in the Schedule of Benefits, so please review that list carefully.
This group health plan is limited to covering preventive and wellness services as required by the Patient Protection and Affordable Care Act as well as other benefits noted in the Summary Plan Description, which describes the benefits covered by the Plan and how these benefits are covered, including information on copays, deductibles, and limitations.

Colorectal Cancer Screening benefit subject to at home test kit for initial screening before benefits are payable for a colonoscopy procedure. If the initial screening test is positive, the plan will provide benefits for a colonoscopy.

GoodHealth Distribution Partners

GHDP-W

Physician Services[1] (Utilizes the First Health Network)[2]	Details		
Wellness Exam	1 Visit / yr	Co-pay	Maximum/ Visit
		$25	$150

Telemedicine	Details	
opyn Live	$0 Consult Fee	No Maximum

BestChoiceRX Participating Pharacies only	Discount Prescriptions Only

Some people need care. Everyone can use support.	Details
Kindly Human™	Hearing from and connecting with someone who can relate to what we're facing is a fundamental human need. When we know we're not alone in what we're facing, everyone feels better. Kindly Human is designed to provide easy access to support 24/7 for everyday challenges that we all face.

GoodHealth Distribution Partners
Plan Sponsor

First Health Network

merchants benefit admin
Plan Administrator

(1) All sickness benefits are subject to a 30-day waiting period before benefits are payable under the plan.
(2) Outpatient physician services and wellness benefits are subject to in-network providers only. Inpatient Hospital indemnity benefits are not.

This Plan does not cover services unless listed in the Schedule of Benefits, so please review that list carefully.
This group health plan is limited to covering preventive and wellness services as required by the Patient Protection and Affordable Care Act as well as other benefits noted in the Summary Plan Description, which describes the benefits covered by the Plan and how these benefits are covered, including information on copays, deductibles, and limitations.

GHDP-1

Physician Services[1] (Utilizes the First Health Network)[2]	Details		
Primary Care Office Visit	3 visits / yr	Co-pay	Maximum / Visit
		$25	$150
Specialist or Urgent Care Office Visit	1 visits / yr	Co-pay	Maximum / Visit
		$50	$300
In-Patient Hospitalization Benefit	$1,000 / Day	$5,000 / Year Maximum	12/12 mo Pre-Ex[3]

Telemedicine	Details	
opyn Live	$0 Consult Fee	No Maximum

BestChoiceRX Participating Pharmacies only	Discount Prescriptions Only

Some people need care. Everyone can use support.	Details
Kindly Human™	Hearing from and connecting with someone who can relate to what we're facing is a fundamental human need. When we know we're not alone in what we're facing, everyone feels better. Kindly Human is designed to provide easy access to support 24/7 for everyday challenges that we all face.

Advocacy	Details
MyHealthcare NINJA	Hospital Bill Reducer

GoodHealth Distribution Partners
Plan Sponsor

First Health Network

merchants benefit admin
Plan Administrator

(1) All sickness benefits are subject to a 30-day waiting period before benefits are payable under the plan.
(2) Outpatient physician services and wellness benefits are subject to in-network providers only. Inpatient Hospital indemnity benefits are not.
(3) Hospitalization benefits are not payable for a Pre-Existing Condition as defined in Section 2.56 Definitions for the first Twelve [12] Months of coverage

This Plan does not cover services unless listed in the Schedule of Benefits, so please review that list carefully.
This group health plan is limited to covering preventive and wellness services as required by the Patient Protection and Affordable Care Act as well as other benefits noted in the Summary Plan Description, which describes the benefits covered by the Plan and how these benefits are covered, including information on copays, deductibles, and limitations.

GoodHealth Distribution Partners

GHDP-2

Physician Services[1] (Utilizes the First Health Network)[2]	Details		
Primary Care Office Visit	4 visits / yr	Co-pay	Maximum/ Visit
		$25	$150
Specialist or Urgent Care Office Visit	2 visits / yr	Co-pay	Maximum / Visit
		$50	$300
In-Patient Hospitalization Benefit	$1,000 / Day	$10,000 / Year Maximum	12/12 mo Pre-Ex[3]

Telemedicine	Details	
opyn Live	$0 Consult Fee	No Maximum

BestChoiceRX Participating Pharmacies only	Details	
Preventive & Acute Prescriptions - (Subject to Formulary - Not subject to a monthly maximum)		
Pharmacy Retail – up to a 30 day supply (Acute & Preventive Generic)	Member Pays	Generic - $0 Copay
Pharmacy Retail up to a 30-day supply or Mailorder up to a 90-day supply. (200 Generic Maintenance Drugs)	Member Pays	Preferred Generic - $5 Copay

Prescription Terms & Conditions
RX Plan includes discounts when the prescription is off of the formulary. Specialty drugs are not covered but the RX provider offers a Prescription Assistance Program - Member must qualify for PAP according to income guidelines. Mail order is optional for generic and brand drugs.

Some people need care. Everyone can use support.	Details
Kindly Human™	Hearing from and connecting with someone who can relate to what we're facing is a fundamental human need. When we know we're not alone in what we're facing, everyone feels better. Kindly Human is designed to provide easy access to support 24/7 for everyday challenges that we all face.

Advocacy	Details
MyHealthcare NINJA	Hospital Bill Reducer

GoodHealth Distribution Partners
Plan Sponsor

First Health Network

merchants benefit admin
Plan Administrator

(1) All sickness benefits are subject to a 30-day waiting period before benefits are payable under the plan.
(2) Outpatient physician services and wellness benefits are subject to in-network providers only. Inpatient Hospital indemnity benefits are not.
(3) Hospitalization benefits are not payable for a Pre-Existing Condition as defined in Section 2.56 Definitions for the first Twelve [12] Months of coverage

This Plan does not cover services unless listed in the Schedule of Benefits, so please review that list carefully.
This group health plan is limited to covering preventive and wellness services as required by the Patient Protection and Affordable Care Act as well as other benefits noted in the Summary Plan Description, which describes the benefits covered by the Plan and how these benefits are covered, including information on copays, deductibles, and limitations.

GoodHealth Distribution Partners

GHDP-3

Physician Services[1] (Utilizes the First Health Network)[2]	Details		
Primary Care Office Visit	4 visits / yr	Co-pay	Maximum/ Visit
		$25	$150
Specialist or Urgent Care Office Visit	4 visits / yr	Co-pay	Maximum / Visit
		$50	$300
In-Patient Hospitalization Benefit	$1,000 / Day	$15,000 / Year Maximum	12/12 mo Pre-Ex[3]
Telemedicine	**Details**		
opyn Live	$0 Consult Fee		No Maximum
BestChoiceRX Participating Pharmacies only	**Details**		
Preventive & Acute Prescriptions - (Subject to Formulary - Not subject to the monthly maximum)			
Pharmacy Retail – up to a 30 day supply (Acute & Preventive Generic)	Member Pays		Generic - $0 Copay
Pharmacy Retail up to a 30 day supply or Mailorder up to a 90-day supply. (200 Generic Maintenance Drugs)	Member Pays		Preferred Generic - $5 Copay
Non-Prefered Generic	Member Pays		Retail 30-day $5 & $10 Copay Mail Order 90-day $5 & $20 Copay
Brand (Prior Authorization Required)	Member Pays		Retail 30-day $40, Mail Order 90-day $80

Prescription Terms & Conditions

Non-Preventive Maintenance Prescriptions - (All available generic and brand drugs. Specialty drugs are not covered). For all non-preventive generic & brand name drugs there is $150 benefit limit per person per month. RX Plan includes discounts when the monthly benefit limit of $150 per person is exceded. Specialty drugs are not covered but the RX provider offers a Prescription Assistance Program - Member must qualify for PAP according to income guidelines. Mail order is optional for generic and brand drugs.

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Advocacy	**Details**
MyHealthcare NINJA	Hospital Bill Reducer

GoodHealth Distribution Partners
Plan Sponsor

First Health Network

mba merchants benefit admin
Plan Administrator

(1) All sickness benefits are subject to a 30-day waiting period before benefits are payable under the plan.
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(3) Hospitalization benefits are not payable for a Pre-Existing Condition as defined in Section 2.56 Definitions for the first Twelve [12] Months of coverage

This Plan does not cover services unless listed in the Schedule of Benefits, so please review that list carefully.
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GoodHealth Distribution Partners

GHDP-4

Physician Services[1] (Utilizes the First Health Network)[2]	Details		
Wellness Exam	1 Visit / yr	Co-pay	Maximum/ Visit
		$25	$150
Primary Care Office Visit	4 visits / yr	Co-pay	Maximum/ Visit
		$50	$150
Specialist or Urgent Care Office Visit	4 visits / yr	Co-pay	Maximum / Visit
		$75	$300
In-Patient Hospitalization Benefit	$1,000 / Day	$10,000 / Year Maximum	12/12 mo Pre-Ex[3]
In/Outpatient Surgery	$1,000 / Year	$2,000 / Year Maximum	12/12 mo Pre-Ex[3]
Emergency Room (if admitted)	$1,000/Per Incident		12/12 mo Pre-Ex[3]
Ambulance Benefit (if admitted)	$500/Per Incident		12/12 mo Pre-Ex[3]

Telemedicine	Details	
opyn Live	$0 Consult Fee	No Maximum

BestChoiceRX Participating Pharmacies only	Details	
Preventive & Acute Prescriptions - (Subject to Formulary - Not subject to the monthly maximum)		
Pharmacy Retail – up to a 30 day supply (Acute & Preventive Generic)	Member Pays	Generic - $0 Copay
Pharmacy Retail up to a 30 day supply or Mailorder up to a 90-day supply. (200 Generic Maintenance Drugs)	Member Pays	Preferred Generic - $5 Copay
Non-Prefered Generic	Member Pays	Retail 30-day $5 & $10 Copay Mail Order 90-day $5 & $20 Copay
Brand (Prior Authorization Required)	Member Pays	Retail 30-day $40, Mail Order 90-day $80

Prescription Terms & Conditions

Non-Preventive Maintenance Prescriptions - (All available generic and brand drugs. Specialty drugs are not covered). For all non-preventive generic & brand name drugs there is $150 benefit limit per person per month. RX Plan includes discounts when the monthly benefit limit of $150 per person is exceded. Specialty drugs are not covered but the RX provider offers a Prescription Assistance Program - Member must qualify for PAP according to income guidelines. Mail order is optional for generic and brand drugs.

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Advocacy	Details
MyHealthcare NINJA	Hospital Bill Reducer

GoodHealth Distribution Partners
Plan Sponsor

First Health Network

mba merchants benefit admin
Plan Administrator

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(3) Hospitalization benefits are not payable for a Pre-Existing Condition as defined in Section 2.56 Definitions for the first Twelve [12] Months of coverage

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GoodHealth Distribution Partners

GHDP-5

Physician Services[1] (Utilizes the First Health Network)[2]	Details		
Wellness Exam	1 Visit / yr	Co-pay	Maximum/ Visit
		$25	$150
Primary Care Office Visit	5 visits / yr	Co-pay	Maximum/ Visit
		$50	$150
Specialist or Urgent Care Office Visit	5 visits / yr	Co-pay	Maximum / Visit
		$75	$300
In-Patient Hospitalization Benefit	$1,500 / Day	$15,000 / Year Maximum	12/12 mo Pre-Ex[3]
In/Outpatient Surgery	$1,500 / Day	$4,500 / Year Maximum	12/12 mo Pre-Ex[3]
Emergency Room (if admitted)	$1,000/Per Incident		12/12 mo Pre-Ex[3]
Ambulance Benefit (if admitted)	$500/Per Incident		12/12 mo Pre-Ex[3]

Telemedicine	Details	
opyn Live	$0 Consult Fee	No Maximum

BestChoiceRX Participating Pharmacies only	Details	
Preventive & Acute Prescriptions - (Subject to Formulary - Not subject to the monthly maximum)		
Pharmacy Retail – up to a 30 day supply (Acute & Preventive Generic)	Member Pays	Generic - $0 Copay
Pharmacy Retail up to a 30 day supply or Mailorder up to a 90-day supply. (200 Generic Maintenance Drugs)	Member Pays	Preferred Generic - $5 Copay
Non-Prefered Generic	Member Pays	Retail 30-day $5 & $10 Copay Mail Order 90-day $5 & $20 Copay
Brand (Prior Authorization Required)	Member Pays	Retail 30-day $40, Mail Order 90-day $80

Prescription Terms & Conditions

Non-Preventive Maintenance Prescriptions - (All available generic and brand drugs. Specialty drugs are not covered). For all non-preventive generic & brand name drugs there is $150 benefit limit per person per month. RX Plan includes discounts when the monthly benefit limit of $150 per person is exceded. Specialty drugs are not covered but the RX provider offers a Prescription Assistance Program - Member must qualify for PAP according to income guidelines. Mail order is optional for generic and brand drugs.

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Advocacy	Details
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GoodHealth Distribution Partners

MEC/Preventive Health Services Summary of Benefits

Preventive Health Services - Covered Benefits[1]

Benefits are automatically subject to 29 CFR § 2590.715 -2713(a). Amendments to this section through legislative act or regulation are automatically incorporated into this document by reference. Preventive Services covered in this section are explained in more detail through the following official resources:

• Medical services with a rating of "A" or "B" from the current recommendations of the United States Preventive Services Task Force. See https://www.uspreventiveservicestaskforce.org

• Preventive care and screenings for infants, children, and adolescents provided for in the comprehensive guidelines supported by the Health Resources and Services Administration. Guidelines can be found in https://www.hrsa.gov

• Immunizations recommended by the Advisory Committee on Immunization Practices of the Centers for Disease Control and Prevention for certain individuals only. See https://www.cdc.gov/vaccines/acip

Benefit	Interval	Requirements
Abdominal Aortic Aneurysm Screening	1 per lifetime	By ultrasonography in men ages 65-75 years who have ever smoked.
Adult Annual Standard Physical	1 per plan year	**Adults**, one (1) physical preventive exam per plan year.
Alcohol Misuse: Unhealthy Alcohol Use Screening and Counseling	1 per plan year	Screenings for unhealthy alcohol use in adults 18 years or older, including pregnant women, and providing persons engaged in risky or hazardous drinking with brief behavioral counseling interventions to reduce unhealthy alcohol use.
Aspirin: Preventive Medication	As prescribed	**Adults** ages 50 to 59 with high risk of cardiovascular diseases and for the primary prevention of cardiovascular disease and colorectal cancer. Low-dose aspirin (81 mg/d) as preventive medication for **women** after 12 weeks of gestation who are at high risk for pre-eclampsia.
Bacteriuria Screening	1 per plan year	Screening for asymptomatic bacteriuria with urine culture in **pregnant women** at 12 to 16 weeks' gestation or at the first prenatal visit, if later.
BRCA Risk Assessment and Genetic Counseling/Testing	1 per plan year	Screening to **women** who have family members with breast, ovarian, tubal, or peritoneal cancer with one of several screening tools designed to identify a family history that may be associated with an increased risk for potentially harmful mutations in breast cancer susceptibility genes (BRCA 1 or BRCA2). Women with positive screening results should receive genetic counseling and, if indicated after counseling, BRCA testing.
Breast Cancer Preventive Medications	As prescribed	Risk-reducing medications, such as tamoxifen, raloxifene, or aromatase inhibitors for women who are at increased risk for breast cancer and at low risk for adverse medication effects.
Breast Cancer Screening	1 time every 2 plan years	Screening mammography for **women** age 50 years and older. Coverage limited to 2D mammograms only.

[1]None of the Preventive Health Services are covered if they are provided at a hospital.

GoodHealth Distribution Partners

MEC/Preventive Health Services Summary of Benefits

Preventive Health Services		
Benefit	**Interval**	**Requirements**
Breastfeeding Support, Supplies and Counseling	In Conjunction with each birth	Interventions during **pregnancy** and **after birth** to support breastfeeding. Costs for renting breastfeeding equipment will be covered in conjunction with each birth.
Cervical Cancer Screening: with Cytology (Pap Smear)	1 time every 3 plan years	**Women** age 21 to 65 years with cervical cytology alone.
Cervical Cancer Screening: with Combination of Cytology and Human Papilloma Virus (HPV) testing	1 time every 5 plan years	**Women** age 30 to 65 years with high-risk papillomavirus (hrHPV) testing alone, or every 5 years with hrHPV testing in combination with cytology.
Chlamydia Screening	1 per plan year	Sexually active **women** age 24 and younger and in older women who are at increased risk infection.
Colorectal Cancer Screening benefit subject to at home test kit for initial screening. If positive, the plan will provide benefits for a colonoscapy.	1 time every 5 plan years	Starting in **adults** at age 50 years and continuing until age 75 years.
Contraceptive Methods and Counseling	As prescribed	Food and Drug Administration (FDA) approved contraceptive methods, sterilization procedures, and patient education and counseling for all women with reproductive capacity, not including abortifacient drugs.
Dental Caries Prevention: Infants and Children Up to Age 5	1 per plan year	Application of fluoride varnish to the primary teeth of all **infants and children** starting at the age of primary tooth eruption and prescription of oral fluoride supplementation starting at age **6 months for children** whose water supply is fluoride deficient.
Depression Screening	1 per plan year	Screening for major depressive disorder (MDD) in **adolescents** aged 12 to 18 years. Screening should be implemented with adequate systems in place to ensure accurate diagnosis, effective treatment, and appropriate follow-up
		Screening for depression in the general **adult** population, including **pregnant and postpartum women**. Screening should be implemented with adequate systems in place to ensure accurate diagnosis, effective treatment, and appropriate follow-up. Pregnant and postpartum persons at increased risk of perinatal depression should be refer to counseling interventions.
Diabetes Screening	1 per plan year	Screening for abnormal blood glucose as part of cardiovascular risk assessment in **adults** aged 40 to 70 years who are overweight or obese. Clinicians should offer or refer patients with abnormal blood glucose to intensive behavioral counseling interventions to promote a healthful diet and physical activity.
Fall Prevention: Older Adults	1 per plan year	Exercise interventions for community-dwelling adults age 65 years and older who are at increased risk for falls.
Folic Acid Supplementation	As prescribed	Daily supplement containing 0.4 to 0.8 mg (400 to 800µg) of folic acid for all **women** planning or capable of pregnancy.
Gestational Diabetes Mellitus Screening	1 per plan year	Asymptomatic **pregnant women** after 24 weeks of gestation.

GoodHealth Distribution Partners

MEC/Preventive Health Services Summary of Benefits

Preventive Health Services		
Benefit	**Interval**	**Requirements**
Gonorrhea Prophylactic Medication	As prescribed	Prophylactic ocular topical medication for all **newborns** for the prevention of gonococcal ophthalmia neonatorum.
Gonorrhea Screening	1 per plan year	Sexually active **women** age 24 years or younger and in older women who are at increased risk for infection.
Healthy Diet and Physical Activity Counseling to Prevent Cardiovascular Disease	1 per plan year	**Adults** who are overweight or obese and have additional cardiovascular disease (CVD) risk factors to intensive behavioral counseling interventions to promote a healthful diet and physical activity for CVD prevention.
Hemoglobinopathies Screening	1 per plan year	Screening for sickle cell disease in **newborns.**
Hepatitis B Screening	1 per plan year	**Non-pregnant** adolescents and adults at high risk for infection.
		Pregnant women at their first prenatal visit.
Hepatitis C Virus (HCV) Infection Screening	1 per plan year	**Persons** at high risk for infection and **Adults born between 1945 and 1965**.
High Blood Pressure Screening	1 per plan year	Screening for high blood pressure in **adults** aged 18 or older.
HIV Preexposure Prophylaxis for the Prevention of HIV Infection	As prescribed	**Persons** who are at high risk of HIV acquisition.
HIV Screening	1 per plan year	**Adolescents and adults** aged 15 to 65 years. Younger adolescents and older adults who are at increased risk should also be screened.
		Pregnant women including those who present in labor, who are untested and whose HIV status is unknown.
Hypothyroidism Screening	1 per plan year	Screening for congenital hypothyroidism in **newborns.**
Intimate Partner Violence Screening	1 per plan year	Screening for intimate partner violence, in **women** of reproductive age and provide or refer women who screen positive to ongoing supporting services.
Lung Cancer Screening	1 per plan year	With low-dose computed tomography in **adults** aged 55 to 80 years who have a 30 pack-year smoking history and currently smoke or have quit within the past 15 years. Screening should be discontinued once a person has not smoked for 15 years or develops a health problem that substantially limits life expectancy or the ability or willingness to have curative lung surgery.
Obesity screening and Counseling	1 per plan year	To **children and adolescents** 6 years and older and offer or refer them to comprehensive, intensive behavioral interventions to promote improvements in weight status.
		Screening all **adults**. Clinicians should offer or refer patients with a body mass index of 30 kg/m2 or higher to intensive, multicomponent behavioral interventions.
Osteoporosis Screening	1 per plan year	In **women** aged 65 and older and in younger postmenopausal **women** at increased risk of osteoporosis.
Phenylketonuria Screening	1 per plan year	Screening for phenylketonuria in **newborns.**

GoodHealth Distribution Partners

MEC/Preventive Health Services Summary of Benefits

Preventive Health Services		
Benefit	**Interval**	**Requirements**
Preeclampsia Screening	1 per plan year	**Pregnant women** with blood pressure measurements throughout pregnancy.
Rh Incompatibility Screening: First Pregnancy Visit	1 per plan year	Rh (D) blood typing and antibody testing for all **pregnant women** during their first visit for pregnancy - related care.
RH Incompatibility Screening: 24–28 Weeks' Gestation	1 per plan year	Repeated Rh (D) antibody testing for all unsensitized Rh (D)-negative **women** at 24 to 28 weeks' gestation, unless the biological father is known to be Rh (D) - negative.
Sexually Transmitted Infections Counseling	1 per plan year	Intensive behavioral counseling for all sexually active **adolescents and for adults** who are at increased risk for sexually transmitted infections.
Skin Cancer Behavioral Counseling	1 per plan year	Counseling young **adults, adolescents, children, and parents of young children** about minimizing their exposure to ultraviolet radiation for persons aged 6 months to 24 years with fair skin types to reduce their risk for skin cancer.
Statin Preventive Medication	As prescribed	**Adults** without a history of cardiovascular disease (CVD) (i.e., symptomatic coronary artery disease or ischemic stroke) use a low- to moderate-dose statin for the prevention of CVD events and mortality when all of the following criteria are met: 1) they are aged 40 to 75 years; 2) they have 1 or more CVD risk factors (i.e., dyslipidemia, diabetes, hypertension, or smoking); and 3) they have a calculated 10-year risk of a cardiovascular event of 10% or greater. Identification of dyslipidemia and calculation of 10-year CVD event risk requires universal lipids screening in adults ages 40 to 75 years.
Syphilis Screening	1 per plan year	**Non-pregnant persons** who are at increased risk for infection.
		All **pregnant women.**
Tobacco Use Counseling and Interventions	2 per plan year	Provide behavioral interventions for cessation to **all adults** who use tobacco, advise them to stop using tobacco, and provide behavioral interventions, U.S. Food and Drug Administration (FDA) approved pharmacotherapy for cessation to adults who use tobacco is covered.
		Provide behavioral interventions for cessation to **pregnant women** who use tobacco.
		Interventions, including education or brief counseling, to prevent initiation of tobacco use in school-aged **children and adolescents.**
Tuberculosis Screening	1 per plan year	Screening for latent tuberculosis infection in **populations** at risk.
Vision Screening	1 time every 2 plan years	**All children** aged 3 to 5 years to detect amblyopia or its risk factors.
Well-Woman Visits	1 per plan year	**Women** under 65 to obtain the recommended preventive services that are age and developmentally appropriate, including preconception care and many services necessary for prenatal care.
Well-Child Visits	1 per plan year	**Children** to obtain the recommended preventive services that are age and developmentally appropriate. (Covers 1 visit except as more frequently recommended for children under the age of 3 years.)

Immunizations

IMMUNIZATIONS - recommended by the Advisory Committee on Immunization Practices of the Centers for Disease Control and Prevention for routine use in children, adolescents, or adults*

Birth Through Six Years Old

Abbreviations	Vaccines	Age Requirements and Limitations
HepB	Hepatitis B	Ages 4 weeks- 2 months
		Ages 6 months- 18 months
DTaP	Diphtheria, tetanus, and acellular pertussis	Ages 15 months- 18 months
Hib	Haemophilus influenzae type b	Ages 12 months- 15 months
PCV13	Pneumococcal 13-valent conjugate	Ages 12 months- 15 months
IPV	Inactivated poliovirus	Ages 6 months-18 months
Flu	Influenza (yearly)	Ages 6 months- 6 years
MMR	Measles, mumps, and rubella	Ages 12 months- 15 months
VAR	Varicella	Ages 12 months- 15 months
HepA	Hepatitis A	Ages 12 months-23 months (1st dose)
		Six months after the last dose (2nd dose)
RV	Rotavirus	Ages 2 months- 6 months (if recommended)

Children From Seven Through Eighteen Years Old

Abbreviations	Vaccines	Age Requirements and Limitations
Flu	Influenza (yearly)	Ages 7 - 18 years
Tdap	Tetanus, diphtheria, and acellular pertussis	Ages 11- 12 years
HPV	Human papillomavirus	Ages 11- 12 years (2 shots series) Note: A 3-shot series of HPV vaccine is needed for those with weakened immune systems and those who start the series at 15 years or older
MenACWY	Meningococcal serogroups A,C,W,Y	Ages 11- 12 years
MenACWY	Meningococcal A,C,W,Y Booster	Age 16 (recommended)

Immunizations

Adults Nineteen Years or Older

Abbreviations	Vaccines	Age Requirements and Limitations
IIV	Influenza inactivated	Ages 19 ≥ 65 years (1 dose annually)
RIV	Influenza recombinant	
LAIV	Influenza live attenuated	Ages 19 - 49 years (1 dose annually)
Tdap	Tetanus, diphtheria, and acellular pertussis	Ages 19 ≥ 65 years (1 dose Tdap, then TD booster every 10 years)
MMR	Measles, mumps, and rubella	Ages 19 - 60 years - 1 or 2 doses depending on indication (if born in 1957 or later)
VAR	Varicella	Ages 19 -37 years - 2 doses (if born in 1980 or later)
RZV	Zoster recombinant	Ages 50 ≥ 65 years - 2 doses
ZVL	Zoster live	Ages 60 ≥ 65 years - 1 dose
HPV - Female	Human papillomavirus	Ages 19 - 26 years - 2 or 3 doses depending on age at initial vaccination
HPV- Male	Human papillomavirus	Ages 19 - 21 years - 2 or 3 doses depending on age at initial vaccination
PCV13	Pneumococcal 13-valent conjugate	Ages ≥ 65 years
PPSV23	Pneumococcal 23-valent polysaccharide	Ages ≥ 65 years

* Immunization illustrations listed herein are based upon CDC recommendations contained in the following schedules: (i) Recommended Child and Adolescent Immunization Schedule (available at: https://www.cdc.gov/vaccines/schedules/hcp/imz/child-adolescent.html), and (ii) Recommended Adult Immunization Schedule (available at: https://www.cdc.gov/vaccines/schedules/hcp/imz/adult.html). Additional immunization scenarios not included in the aforementioned illustrations (such as catch-up immunization recommendations, immunization recommendations for certain high-risk groups, and immunization recommendations subject to individual clinical decision-making) may also be covered under this Plan pursuant to CDC recommendation. Information concerning these additional covered immunization scenarios (including vaccine type, age requirements, and frequency) is available online under the CDC schedule links listed above. Paper copies of these CDC schedules can also be obtained free of charge by written request to the Plan Administrator.

Exclusions

The following exclusions apply to the benefits offered under this Plan:

1. Office visits, physical examinations, immunizations, and tests when required solely for the following:
   a. Sports,
   b. Camp,
   c. Employment,
   d. Travel,
   e. Insurance,
   f. Marriage,
   g. Legal proceedings
2. Routine foot care for treatment of the following:
   a. Flat feet,
   b. Corns,
   c. Bunions,
   d. Calluses,
   e. Toenails,
   f. Fallen arches,
   g. Weak feet,
   h. Chronic foot strain
3. Dental procedures
4. Any other medical service, treatment, or procedure not covered under this Plan
5. Any other expense, bill, charge, or monetary obligation not covered under this Plan, including but not limited to all non-medical service expenses, bills, charges, and monetary obligations. Unless the medical service is explicitly provided by this Appendix A or otherwise explicitly provided in this Plan Document, this Plan does not cover the medical service or any related expense, bill, charge, or monetary obligation to the medical service
6. Claims unrelated to treatment of medical care or treatment
7. Cosmetic surgery unless authorized as medically necessary. Such authorization is based on the following causes for cosmetic surgery: accidental injury, correction of congenital deformity within six (6) years of birth, or as a treatment of a diseased condition
8. Any treatment with respect to treatment of teeth or periodontium, any treatment of periodontal or periapical disease involving teeth surrounding tissue, or structure. Exceptions to this exclusion include only malignant tumors or benefits specifically noted in the schedule of benefits to the Plan Document
9. Any claim related to an injury arising out of or in the course of any employment for wage or profit
10. Claims which would otherwise be covered by a Worker's Compensation policy for which a participant is entitled to benefit

11. Any claim arising from service received outside of the United States, except for the reasonable cost of claims billed by the Veterans Administration or Department of Defense for benefits covered under this Plan and not incurred during or from service in the Armed Forces of the United States
12. Claims for which a participant is not legally required to pay or claims which would not have been made if this Plan had not existed
13. Claims for services which are not medically necessary as determined by this Plan or the excess of any claim above reasonable and customary rates when a PPO network has not been contracted
14. Charges which are or could be reimbursed by any public health program irrespective of whether such coverage has been elected by a participant
15. Claims due to an act of war, declared or undeclared, not including acts of terrorism
16. Claims for eyeglasses, contacts, hearing aids (or examinations for the fitting thereof) or radial keratotomy
17. Abortion Services
18. Travel, unless specifically provided in the schedule of benefits
19. Custodial care for primarily personal, not medical, needs provided by persons with no special medical training or skill
20. Claims from any provider other than a healthcare provider as defined in the Plan Document unless explicitly permitted in the schedule of benefits
21. Investigatory or experimental treatment, services, or supplies unless specifically covered under Approved Clinical Trials
22. Services or supplies which are primarily educational
23. Claims due to attempted suicide or intentionally self-inflicted injury while sane or insane, unless the claim results from a medical condition such as depression
24. Claims resulting from, or which arise due to the attempt or commission of, an illegal act. Claims by victims of domestic violence will not be subject to this exclusion
25. Claims with respect to any treatment or procedure to change one's physical anatomy to those of the opposite sex and any other treatment or study related to sex change
26. Claims from a medical service provider who is related by blood, marriage, or legal adoption to a participant
27. Any claims for fertility or infertility treatment
28. Claims for weight control, weight reduction, or surgical treatment for obesity or morbid obesity, unless explicitly provided in the schedule of benefits
29. Claims for disability resulting from reversal of sterilization
30. Claims for the completion of forms, or failure to keep scheduled appointments
31. Recreational or diversional therapy
32. Personal hygiene or convenience items, including but not limited to air conditioning, humidifiers, hot tubs, whirlpools, or exercise equipment, irrespective of the recommendations or prescriptions of a medical service provider
33. Claims due to participation in a dangerous activity, including but not limited to sky-diving, motorcycle or automobile racing, bungee jumping, rock climbing, rappelling, or hang gliding
34. Claims that arise primarily due to medical tourism
35. Supportive devices of the foot
36. Treatments for sexual dysfunction
37. Aquatic or massage therapy
38. Biofeedback training
39. Skilled nursing facilities
40. Durable medical equipment and prosthetics
41. Hospice care, private duty nursing, or long-term care
42. Residential facility – for charges from a residential halfway house or home, or any facility which is not a health care institution licensed for the primary purpose of treatment of an illness or injury
43. Claims for temporomandibular joint syndrome
44. Claims for biotech or specialty prescriptions
45. Any claim which is not explicitly covered in the schedule of benefits
46. Genetic testing unless explicitly covered in the schedule of benefits
47. Organ transplants
48. Claims for cosmetic surgery, not related to mastectomy reconstruction to produce a symmetrical appearance or prosthesis, or physical complications which result from such procedures.
49. Chiropractic care
50. Radiation and chemotherapy
51. Dialysis
52. Acupuncture
53. Alternative medicine/homeopathy
54. Children dental and vision
55. Neonatal intensive care (NICU)
56. Rehabilitative therapies
57. PCP surgery
58. Routine eye care (Adult)
59. Non-emergency care when traveling outside the U.S.
60. Pregnancy Benefits, including office visits and childbirth/delivery professional and facility services.
61. Routine well-baby care of newborn infant while inpatient.

"The purpose of this list of exclusions is solely to provide additional clarity regarding treatments, procedures, products, services, or any other items which are not covered under this plan. Accordingly, no exclusion shall be interpreted by negative implication, or otherwise, as evidence of the existence of coverage under this plan."

Real Benefits.
Real Value.

GoodHealth
Distribution Partners

Individual & Family Plans

GHD,LP Dental is a self-funded PPO plan that utilizes the MaximumCare PPO networks including Careington, Connection Dental, and Dentemax. To locate a provider, Click Here.

Dental 1500	Dental 3000	Dental 5000
▶ $1,500 annual max per person	▶ $3,000 annual max per person	▶ $5,000 annual max per person

- $50 annual deductible for basic and major services (per person)
- $150 max (per family)
- No deductible for preventative services.

PREVENTIVE CARE (100% Coverage*) No Waiting Period

- Routine Exam (2 per 12 months)
- Bitewing X-rays (1 per 12 months)
- Cleaning (2 per 12 months)
- Fluoride for children under age 16 (1 per 12 months)

BASIC CARE (80% Coverage*) No Waiting Period

- Full Mouth/Panoramic X-rays (1 per 3 years)
- Sealants (ages 6 through 16)
- Space Maintainers (child under 16)
- Restorative Amalgams (fillings)
- Simple Extractions

MAJOR CARE (50% Coverage*) 12 Month Waiting Period

- Onlays
- Oral Surgery
- Crowns (1 per tooth, per 7 years)
- Crown Repair
- Endodontics (nonsurgical)
- Periodontics (nonsurgical)
- Periodontics (surgical)
- Denture Repair
- Dentures (1 appliance per 5 years)
- Bridge (1 per 7 years)
- Complex Extractions
- Anesthesia

Products are not available in all states, including: MA, MD, ME, NH, VT, WA.

This plan follows the "Carve Out" Coordination of Benefits method. This means that it first calculates the normal plan benefits that would be paid, then reduces this amount by the amount paid by the primary plan.

*Limitations & Exclusions

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1. No Benefits are payable under the Policy for the Services listed below. In addition, the Services listed below will not be recognized toward the satisfaction of any Deductible:

2. Any Services which are not included in the Schedule of Covered Procedures;

3. Any Service started or appliance installed before the Effective Date or after the Termination Date, except in those instances noted in this Certificate;

4. Any Service, which may not reasonably be expected to successfully correct the patient's dental condition for a period of at least 5 years, as determined by Us;

5. Any procedure We determine is not necessary, does not offer a favorable prognosis, does not have uniform professional endorsement or is experimental in nature;

6. Any treatment which is elective or primarily cosmetic in nature and not generally recognized as a generally accepted dental practice by the American Dental Association, as well as any replacement of prior cosmetic restorations unless such procedure is listed in the Schedule of Covered Procedures;

7. Appliances, Services or procedures relating to:
- *a. the change or maintenance of vertical dimension;*
- *b. splinting;*
- *c. correction of attrition, abrasion, erosion, or abfraction;*
- *d. bite registration; or*
- *e. bite analysis;*

8. Replacement of bridges unless the bridge is older than the age allowed in the Schedule of Covered Procedures and cannot be made serviceable;

9. Replacement of full or partial dentures unless the prosthetic appliance is older than the age allowed in the Schedule of Covered Procedures and cannot be made serviceable;

10. Replacement of crowns, inlays or onlays unless the prior restoration is older than the age allowed in the Schedule of Covered Procedures and cannot be made serviceable;

11. For Orthodontia Services;

12. Services provided for any type of temporomandibular joint (TMJ) dysfunctions, muscular, skeletal deficiencies involving TMJ or related structures, myofascial pain unless such procedure is listed as a Covered Procedure in the Schedule of Covered Procedures;

13. Charges for precision or semi-precision attachments, denture duplication, overdentures and any associated surgery, or other customized Services or attachments;

14. Athletic mouth guards; myofunctional therapy; treatment for malignancies, cysts and neoplasms; failure to keep scheduled appointment; charges for completion of Claim forms, infection control; precision or semi-precision attachments; denture duplication; oral hygiene instruction; separate charges for acid etch; charges for travel time; transportation costs; professional advice; treatment of jaw fractures; orthognathic surgery; exams required by a third party other than Us, personal supplies (e.g., water pik, toothbrush, floss holder, etc.); or replacement of lost or stolen appliances; augmentation and socket preservation, tissue and bone grafting, implant connecting bars and supporting structures.

15. Prescription drugs, premedication, pharmaceuticals, or analgesia;

16. Dental disease, defect or injury caused by a declared or undeclared war or any act of war or terrorism or taking part in an insurrection or riot; the commission or attempted commission of a crime; an intentionally self-inflicted injury or attempted suicide while sane or insane;

17. Dental treatment not approved by the American Dental Association or which is clearly experimental in nature;

18. Any charge for a Service for which benefits are available under Worker's Compensation or an Occupational Disease Act or Law, even if You did not purchase the coverage that is available to You;
20. Any charge for a Service performed outside of the United States other than for Emergency Treatment. Benefits for Emergency Treatment performed outside of the United States are limited to a maximum of $100 per Plan Year.

19. The initial placement of a removable full denture or a removable partial denture unless it includes the replacement of a Natural Tooth extracted while the Person is insured under the Policy;

20. The initial placement of a fixed partial denture including a Maryland Bridge, unless it includes the replacement of a Natural Tooth extracted while the Person is insured under the Policy, provided that tooth was not an abutment to an existing partial denture. Frequency Limitations for replacement of Dentures and bridges are stated in the Schedule of Covered Procedures. Benefits are payable only for the replacement of those teeth which were extracted while the Person was insured under the Policy;

21. The replacement of teeth beyond the normal complement of 32;

22. The replacement of an existing removable partial denture with a fixed partial denture unless upgrading to a fixed partial denture is essential to the correction of the Covered Person's dental condition;

23. Local, including light anesthetic, as a separate fee;

24. Any Treatment Plan which involves full-mouth reconstruction by the removal and reestablishment of occlusal contacts of 10 or more teeth with restorations, crowns, onlays, inlays, fixed partial dentures, dentures, or any combination of these Services;

25. Services with respect to congenital (hereditary) or developmental (before birth) malformations, except during the 31 day period immediately following the birth of Your Child, including but not limited to; cleft palate, maxillary and mandibular (upper and lower) malformations, enamel hypoplasia (lack of development), fluorosis, and anodontia;

26. Dental care paid for, required, or provided by or under the laws of a national, state, local or provincial government, or treatment furnished within a hospital or other facility owned or operated by a national or state government unless the Insured Person has a legal obligation to pay;

27. Dental services performed in a hospital and related hospital fees;

28. Services covered under an existing medical plan;

29. The portion of an expense which is in excess of the reasonable charge;

30. Fees associated with a cancelled or missed appointment;

31. Nitrous Sedation

*Limitations & Exclusions

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Alternate Benefits: *There is often more than one Service that can be used to treat a dental problem or disease. In determining the Benefits payable on a Claim, different materials and methods of treatment will be considered. The amount payable will be limited to the Covered Expense for the least costly Service, which meets broadly accepted standards of dental care as determined by us. The Covered Person and his or her Provider may decide on a more costly procedure or material than We have determined to be satisfactory for the treatment of the condition. We will pay a benefit toward the cost of the more expensive procedure or material, but payment will be limited to the Benefits Payable for Covered Expenses for the least costly Service. We will not pay the excess amount.*

Resin fillings on posterior teeth (molars and bicuspid/premolar) will be paid at the rate for amalgam fillings. Resin fillings on anterior teeth will be paid at the rate for amalgam fillings when lingual (indicated as surface code "L") as an access surface for restoration placement is inclusive in the restoration. Porcelain crown/bridge units on posterior teeth will be paid at the rate of the full cast crowns/bridge units.

Missing Tooth Clause Limitation: We will not pay benefits for replacement of teeth missing on a Covered Person's Effective Date of insurance for the purpose of the initial placement of a full denture, partial denture, or fixed bridge. In addition, such replacement will not be recognized toward the satisfaction of any Deductible. However, expenses for the replacement of teeth missing on the Effective Date will be considered for payment as follows:

1. The initial placement of full or partial dentures will be considered a Covered Procedure if the placement includes the initial replacement of a Natural Tooth extracted while the Covered Person is covered under the **Group Policy.**

2. The initial placement of a fixed bridge will be considered a Covered Procedure if the placement includes the initial replacement of a Natural Tooth extracted while a Covered Person is covered under the policy. However, the following restrictions will apply:

a. Benefits will only be paid for the replacement of the teeth extracted while a Covered Person is covered under the Group Policy

b. Benefits will not be paid for the replacement of other teeth which were missing on the Covered Person's Effective Date.

c. Missing teeth limitation will be waived after a Covered Person has been covered under the plan for 5 continuous years unless it is a replacement of an existing unserviceable prosthesis.

Timely filing limit: A clean claim (meaning no missing or incorrect numbers or information) must be submitted to us within 12 months, or one calendar year, from the date of service. Claims will be denied if they arrive after the deadline date.

Coordination of Benefits (COB)

When a Covered Person has dental coverage under more than one Plan, as defined below, the benefits payable between the Plans will be coordinated.

Benefit Coordination: This policy follows the "Carve Out" Coordination of Benefits method. This means that it first calculates the normal plan benefits that would be paid, then reduces this amount by the amount paid by the primary plan.

Order of Benefit Determination:
1. When this is the Primary Plan, we will pay Benefits as if there were no other Plans.

2. When a person is covered by a Plan without a COB provision, the Plan without the provision will be the Primary Plan.

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3. When a person is covered by more than one Plan with a COB provision, the order of benefit payment is as follows:

a. Non-Dependent/Dependent. A Plan that covers a person other than as a Dependent will pay before a Plan that covers that person as a Dependent.

b. Dependent Child/Parents Not Separated or Divorced. For a Dependent Child, the Plan of the parent whose birthday occurs first in the Calendar Year will pay benefits first. If both parents have the same birthday, the Plan that has covered the Dependent Child for the longer period will pay first.

c. Dependent Child/Separated or Divorced Parents. If two or more Plans cover a person as a Dependent of separated or divorced parents, benefits for the Child are determined in the following order:

i. The Plan of the parent who has responsibility for providing insurance as determined by a court order.
ii. The Plan of the parent with custody of the Child.
iii. The Plan of the Spouse of the parent with custody; and
iv. The Plan of the parent without custody of the Child.

d. Dependent Child/Joint Custody: If the joint custody court decree does not specifically state which parent is responsible for the Child's medical expenses, the rules as shown for Dependent Child/Parents Not Separated or Divorced shall apply.

e. Longer/Shorter Length of Coverage. When an order of payment is not established by the above, the Plan that has covered the person for the longer period of time will pay first.

Right to Receive and Release Needed Information: We may release to, or obtain from, any other insurance company, organization, or person information necessary for COB. This will not require the consent of, or notice of, you or any claimant. You are required to give us information necessary for determining COB.

Right to Make Payments To Another Plan: COB may result in payments made by another Plan that should have been made by us. We have the right to pay such other Plan all amounts it paid which would otherwise have been paid by us. Amounts so paid will be treated as benefits paid under this Plan. We will be discharged from liability to the extent of such payments.

Right to Recovery: COB may result in overpayments by us. We have the right to recover any excess amounts paid from any person, insurance company or other organization to whom, or for whom, payments were made.

Appeal Process: If We send you a Written statement denying your claim in whole or in part, you may submit a written appeal to us that outlines your concerns and your efforts to resolve the matter including the date(s) of service and claim number(s). The appeal must be filed within 60 days of the receipt of denial. A Written decision with respect to the appeal shall be sent to you within 15 days after its receipt, unless special circumstances exist which require additional time, in which case a written decision with respect to the appeal will be sent to you as soon as possible.

Please send to:
Merchants Benefit Administration
Attn: Appeals Department,
18700 N Hayden Rd, Ste 390
Scottsdale, AZ 85255

If You are not satisfied by the appeal response or for any reason, please refer to the plan document.

Smarter Vision Care

GoodHealth Distribution Partners

A HASSLE FREE APPROACH TO VISIONCARE

Traditional vision insurance hasn't evolved in 20+ years, but your people's needs have.

Limited access: 50%
Employees who use their vision insurance

High costs: $230/pair
Even with insurance, employees pay an average of $230/pair

Poor experience: -8 NPS
Vision insurers have low member satisfaction

GWSW Vision is different. Our digital-first platform easily replaces traditional vision insurance while keeping costs low and satisfaction high. XP Health provides:

AN EYEWEAR MARKETPLACE WITHOUT THE MARKUPS

ACCESS FOR ALL 100% USAGE

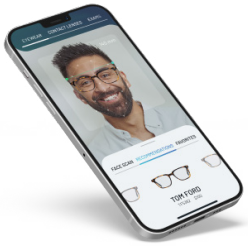
With XP Health, even those with 20/20 vision can use their benefits for sunglasses, blue light glasses, and more

LOWER COSTS $42/PAIR

Offer 1,500+ designer frames & sunglasses at an average cost of $42/pair

GREAT EXPERIENCE +72 NPS

Our online marketplace uses intuitive AI technology and virtual try-on to find the perfect eyewear



IMPROVED ACCESS TO EYE EXAMS

In-person eye exams for $0

Employees can choose from 99,000+ providers at private practices & national, regional, and local retail chains.

RX renewals on demand

Employees can get the care they need through simple online prescription renewals.

TRUSTED AND VETTED

We're partnering with innovative organizations and recognized by top publications.

*This plan is only available when bundled with one of 3 dental plans. (1500 Plan, 3000 Plan, 5000 Plan)

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